

Better Together?

The Effects of Integrated Social Services for Women

M. BUSTELO

S. MARTINEZ

M. PÉREZ

J. RODRÍGUEZ

Inter-American
Development Bank

International Initiative for
Impact Evaluation

World Health
Organization

University of Oxford

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Abstract

We study the effects of integrated social services for women in El Salvador using a randomized experiment. The Ciudad Mujer ‘one-stop shop’ centers provided 30 public services including health, gender-based violence, legal, financial, and employment services on a single campus accessible only to women. These integrated services could boost demand by reducing the cost of access, improving quality and exploiting complementarities in service provision. Over 4,000 women were randomly encouraged to visit Ciudad Mujer (treatment group) or a local health clinic (placebo group), or they received no encouragement (control group). Approximately 1 year later, women exposed to Ciudad Mujer visited the center an additional 2.1 times, increased the use of public services by 0.53 s.d. and they reported a 10% improvement in life satisfaction relative to both the placebo and control groups.

JEL Codes: I38, J1, C93

Keywords: Ciudad Mujer, one-stop shop, randomized promotion, public service provision, gender equality.

Monserrat Bustelo: monserratb@iadb.org
Sebastian Martinez: smartinez@3ieimpact.org
Michelle Pérez: miperez@who.int
Julio Rodríguez: julio.rodriguez@bsg.ox.ac.uk

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This study was conducted in partnership with the Government of El Salvador. The Ministry of Social Inclusion (SIS) was responsible for the *Ciudad Mujer* program, while the data collection for the impact evaluation was led by the former General Directorate of Statistics and Censuses (DIGESTYC), which was, by law, the governing entity coordinating the national statistical system and conducting research on economic, demographic, and social topics. As *Ciudad Mujer* was a strategic program of the administration, both SIS and DIGESTYC were responsible for disseminating the program's impact results.

As part of the Ministry's political agenda and the Directorate's special projects research agenda, the authors were informed that the data collection did not require an Institutional Review Board review but still needed to adhere to all procedural and ethical standards established by the entities, including data confidentiality and the use of informed consent forms. These forms were indeed collected and stored in accordance with the Offices' requirements.

1. Introduction

Improving access to public services that address women's needs is crucial. In 2020, the maternal mortality rate in Latin America and the Caribbean (LAC) was 50 deaths per 100,000 women, and the adolescent pregnancy rate was among the highest globally (Liang et al., 2019). Despite the rise in female labor force participation to 61% over the past 50 years, significant gender disparities persist. Women are overrepresented in lower-paying, informal sectors and earn 9% to 25% less than men with similar education levels (Frisancho and Queijo, 2022; Urquidi and Chalup, 2023). Additionally, gender-based violence affects one-quarter of women aged 15-49 in LAC, with severe consequences for their health and well-being (WHO, 2021). These persistent challenges are closely linked to structural barriers that limit women's ability to access essential public services.

Access barriers for vulnerable groups, such as low-income women, remain high, leading to the underutilization of critical social services. While governments have traditionally focused on demand-side solutions, innovative supply-side models like One-Stop Shops (OSS) aim to improve service accessibility and satisfaction by consolidating services in one location (Fredriksson, 2020; Pfeil et al., 2017; Post and Agarwal, 2011). OSS models vary widely in design and purpose—from administrative efficiency reforms (e.g., citizen service centers in Canada and India) to social welfare platforms providing comprehensive assistance. Most of these initiatives focus on streamlining bureaucracy or improving user satisfaction, rather than addressing gender-specific barriers to service access (Askim et al., 2011; Dean, 2011; OECD, 2020). In addition, empirical evaluations are scarce, and almost none target women's empowerment or well-being as a primary program goal (Fredriksson, 2020; Colombini et al., 2020; Bell, 2022).

In this study, we evaluate the effectiveness of *Ciudad Mujer* (CM) centers, an OSS model specifically designed for women. Using a randomized encouragement design, we focus on its

impact on service utilization and women's well-being. Additionally, the analysis leverages rich panel survey data and administrative records, enabling a detailed exploration of program effectiveness beyond user satisfaction. This is the first rigorous evaluation of an OSS model specifically designed for women.

The findings show that one year after the intervention, more than 60% of women in the treatment group visited CM, compared to 10% in the placebo and control groups. We find a 0.53 standard deviations increase in service use for treated women, particularly in reproductive and sexual health, legal services, and economic autonomy services. Older and less-educated women benefitted the most, suggesting that CM effectively targets the most vulnerable populations. Additionally, women in the treatment group were 8.5 percentage points more likely to report being satisfied with their lives, a 10% increase over the comparison groups. These results highlight the importance of integrated service delivery models in addressing gender-specific barriers to public service access, with significant policy implications for the LAC region and potentially other developing areas.

Importantly, the CM model combines several mutually reinforcing features—including co-location of multiple public services in one facility, integrated case management, women-only staffing, gender-sensitive service delivery, and on-site childcare—but we are not able to isolate the effect of any single component, and therefore the estimated effects reflect the impact of CM as a whole.

Ciudad Mujer represents a distinct, gender-responsive adaptation of the OSS model. Launched in El Salvador in 2011, it sought to reduce institutional fragmentation and increase women's access to public services in low-income areas by consolidating sexual and reproductive health, legal aid, violence prevention, and economic empowerment services under one roof. Its design integrates a

women-only environment, gender-sensitive protocols, and on-site childcare, ensuring safety and inclusion. Unlike traditional OSS reforms, CM explicitly mainstreams a gender perspective in service design, staffing, and delivery. It thus aligns more closely with multisectoral models of women's empowerment—similar to *Women's Justice Centers* or *Integrated Support Centers* in Latin America—than with administrative efficiency initiatives (CEPAL, 2019; Bell, 2022).

Studies on OSS generally examine institutional and political aspects of service integration or document implementation experiences (Askim et al., 2011; Bussell, 2010; Dean, 2011; Esteves de Araújo, 2001; Fredriksson, 2020). Canada implemented OSS reforms in the late 1990s, but the absence of pre-implementation data prevented rigorous evaluation (Bent et al., 1999). In Russia, Gallagher et al. (2003) found that OSS users saved between 1.3 and 2.4 hours compared to traditional service structures. In India, Bussell (2010) evaluated the *Nemmadi Kendras* initiative, showing that computerized one-stop centers reduced the number of visits but not total time spent. Fredriksson (2020) used a difference-in-differences approach to estimate the impact of Brazil's *Poupatempo*, finding substantial time savings but mixed results on service quality.

Beyond administrative reforms, a smaller set of OSS initiatives have been designed to improve women's access to essential services and promote empowerment. In Africa and Asia, *One-Stop Centers* for survivors of gender-based violence integrate medical, psychosocial, and legal services (Colombini et al., 2012; Colombini et al., 2020; García-Moreno et al., 2015). The 2022 GBV AoR Helpdesk review documented similar models in over 15 countries, highlighting Latin American 'Women's Justice Centers' and *Ciudad Mujer* as distinctive examples that combine protection, health, and economic support within a single institutional platform (Bell, 2022). However, rigorous impact evaluations of such gender-focused integrated models remain virtually absent, leaving open

questions about their effectiveness in improving women’s well-being and empowerment outcomes (Buvinic & O’Donnell, 2020; Klugman et al., 2022).

This paper contributes to the limited body of literature by estimating the causal effect of an OSS model, specifically analyzing the overall impact of CM on women’s service use and well-being. Unlike many OSS models, CM is designed for women’s needs, offering insights into gender-specific service adaptations and marking this study as the first rigorous evaluation of such a model.

The remainder of the paper is organized as follows. Section 2 discusses the conceptual framework behind integrated public delivery models. Section 3 describes the CM model. Section 4 outlines the encouragement design, and Section 5 describes the data sources. Section 6 explains the empirical identification strategy, and Section 7 presents the main results. Section 8 concludes our findings.

2. Conceptual Framework

Social services, such as health care, policing, and employment programs, are typically provided or financed by governments to address societal challenges and improve population well-being (Barr, 2019; Hindriks & Myles, 2013; Stiglitz & Rosengard, 2015). These services aim to enhance social welfare, even if benefits are privately captured. For instance, health services prevent and treat illness, improving quality of life; policing reduces violence and crime; and employment services expand job opportunities and incomes (Card et al., 2018; Levitt, 2004; World Bank, 2004). Public provision of these services promotes equity and internalizes positive spillovers that the private market may not capture, such as health benefits from child immunization or the social value of reduced crime (Desmarais-Tremblay, 2017; Musgrave, 1959).

In many countries, the provision of public services remains fragmented both geographically and organizationally, with multiple agencies serving the same population independently. This fragmentation often forces beneficiaries to visit several offices to access interrelated services incurring significant transaction costs such as time, transportation, childcare and safety risks (Dunleavy & Carrera, 2013; OECD, 2023; World Bank, 2004). Fragmented service delivery also leads to variations in quality and responsiveness and may fail to address gender-specific needs (Colombini et al., 2020; García-Moreno et al., 2015). Moreover, institutional fragmentation weakens complementarities and coordination across programs, reducing overall efficiency and accountability in service provision (Greene et al., 2021; Strokosch & Bovaird, 2025).

Access barriers are particularly high for vulnerable groups, such as low-income women, who often underutilize critical social services like prenatal health care or legal assistance for gender-based violence¹ (World Bank, 2004). Governments have traditionally focused on demand-side solutions, such as conditional cash transfers, to boost service utilization (Fiszbein and Schady, 2009). However, innovative supply-side models aimed at reducing access barriers and increasing utilization have received less attention.

In recent decades, government modernization initiatives have increasingly adopted mechanisms to provide integrated public services (Kernaghan, 2009). OSS models aim to enhance accessibility and user satisfaction while reducing administrative costs by consolidating multiple agencies and services in a single location (Dean, 2011). These models represent a shift from fragmented public administration toward a citizen-oriented structure (Wimmer, 2002), bringing

¹ Gender-based violence is defined as any action or omission based on gender that causes a woman's death, injury, physical, sexual or psychological suffering and moral or patrimonial damage (UN Women, 2010). Definition of domestic violence: <http://www.endvawnow.org/en/articles/398-definition-of-domestic-violence.html> (Accessed on 10 June 2024).

services together to reduce the time and effort required to access. OSS models have the potential to improve accessibility, convenience, and overcome jurisdictional divisions (Bent et al., 1999).

Countries such as Canada and Australia are recognized pioneers in integrating siloed transactional services into common counters (Dean, 2011). The United Kingdom has implemented OSS approaches for sexual health services (Griffiths et al., 2008), while many OECD countries have adopted it in the welfare sector (Askim et al., 2011). In the context of gender-based violence, OSS centers—most commonly located within hospitals or as stand-alone facilities—offer integrated medical, legal, and psychosocial care for women and girls (Colombini et al., 2012; Ellsberg et al., 2015; GBV AoR Helpdesk, 2022, OECD, 2023).

The rationale behind OSS centers rests on reducing transaction costs, improving service quality through complementarities, and focusing on beneficiaries' needs. First, consolidating services helps lower costs for users, providers, and the government by minimizing duplication and effort (Askim et al., 2011). Second, collaboration between public agencies enhances access to complementary services and strengthens coordination, improving service quality. Finally, OSS centers align services with the complete beneficiaries' life trajectories, addressing bureaucratic silos and enabling integrated, person-centered care through a single entry point (PWC, 2012; Strokosch & Bovaird, 2025).

Despite the growing number of OSS initiatives worldwide, the concept remains underexplored in the empirical literature, with limited evidence on its effectiveness in improving access or outcomes. *Ciudad Mujer* builds on the OSS approach but advances it by integrating a gender-responsive framework that addresses women's multidimensional barriers to service access. This paper contributes to filling this gap by providing rigorous evidence on the short-term effects of this integrated, women-centered model on service utilization and well-being.

3. Ciudad Mujer (CM)

The CM program aimed to improve public service delivery for women by providing key services through OSS centers. The program sought to enhance gender equality by offering an integrated response to women's needs in sexual and reproductive health, economic autonomy, and gender-based violence prevention and care. CM centers provided a comprehensive range of free public services grouped into five modules: (1) sexual and reproductive health, (2) economic autonomy, (3) gender-based violence, (4) collective education and (5) childcare.

The sexual and reproductive health module included a medical team specializing in gynecology, breast and cervical-uterine cancer prevention, sexually transmitted diseases, and family planning. The economic autonomy module supported employment and small and micro enterprises through labor intermediation services, job training sessions, financial education and microcredits. The gender-based violence module offered medical and psychological care, legal assistance, and police protection for victims, addressing physical, sexual, emotional, and patrimonial violence.² The collective education module provided educational services that reached out to nearby communities to promote women's rights and prevent gender-based violence. Additionally, the centers offered childcare for children up to 12 years old while the mother or caregiver used the facility's services and hosted an on-site cafeteria with food for purchase.

² Physical violence is understood as any behavior that offends a woman's bodily integrity or health. Sexual violence is understood as any behavior that forces a woman to witness, maintain or participate in unwanted sexual intercourse by means of intimidation, threat, coercion or the use of force. Emotional violence is understood as any behavior that causes emotional damage and reduction of self-esteem, that harms and disturbs full development or that aims at degrading or controlling a woman's actions, behaviors, beliefs and decisions. Patrimonial violence is understood as any behavior that constitutes retention, subtraction, partial or full destruction of a woman's objects, working instruments, personal documents, property, assets and economic rights or resources, including those intended to satisfy her needs (UN Women, 2010). Definition of domestic violence: <http://www.endvawnow.org/en/articles/398-definition-of-domestic-violence.html> (Accessed on 10 June 2024).

Overall, the centers offered more than 30 services, integrating 18 public agencies in a single setting.

The CM program's multifaceted intervention included several components beyond reduced transportation costs and better coordination among services. The gender-specific design, featuring female-only staff and clientele, on-site childcare, and comprehensive service integration, created a unique and supportive environment. This holistic approach addressed multiple barriers to service utilization simultaneously. All services at the CM centers were designed specifically for women. First, all staff members were women, and only women could access the centers. Second, all staff were trained to listen to and treat beneficiaries within a rights-based and gender-sensitive framework. Third, each case was managed with an integrated approach, offering personalized service and one-time-only registration to reduce waiting times and access costs. Upon arrival, women received personalized attention from a counselor who assessed their situation in terms of health, family relationships, labor status, and safety at home and in the community, and prepared a roadmap to help them access the services that best meet their needs.

All services were free for beneficiaries, open to the public, and provided on a first-come, first-served basis. According to the internal administrative cost report by El Salvador's Ministry of Social Inclusion (Secretaría de Inclusión Social, 2013, unpublished), the estimated operational cost per service at Ciudad Mujer centers ranged from US\$12 to US\$24, depending on the location. The San Martin center, for example, provided 114,284 services in 2013 at a total operational cost of US\$2.74 million, resulting in an average of US\$12 per service. These figures are based on detailed disaggregation by cost type—distinguishing *direct costs* (mainly personnel expenses directly associated with service provision) and *indirect costs* (such as utilities, maintenance, and

administrative support)—as well as by service module (health, gender-based violence, economic autonomy, education, and childcare). All estimates exclude pre-operation capital expenditures.

Between 2011 and 2016, El Salvador’s Secretary of Social Inclusion launched six centers across the country.³ While services were open to all Salvadorian women, the intended catchment area was the municipalities contiguous to the centers. During the study period, the CM program served 485,000 women, representing approximately 14% of the country’s female population.⁴

4. Encouragement Design

CM has an ‘open door’ policy of providing services to all women who access the center on a first-come, first-served basis. Given that participation is likely to be correlated with unobservable characteristics, the identification of treatment effects requires a valid instrument to correct for endogenous selection into treatment. To this effect, we implemented a randomized encouragement design whereby women between the ages of 18 and 60 years, living in the geographical catchment area of three new CMs, would be offered an incentive to visit the nearest CM, the nearest health center (placebo) or no promotion (control). The promotion campaign for women assigned to the CM and placebo groups was framed as a ‘women's health campaign,’ and to minimize differences in the encouragement messaging, they were encouraged to visit the health centers at either CM or the nearest health center/unit. Health centers were located within the same municipalities as the

³ The six centers were in the departments of San Salvador, La Libertad, Santa Ana, San Miguel, Usulután and Morazán. The centers’ locations were identified using three criteria: maximization of female population coverage, availability of public land or property that could be used for building the facilities and the accessibility for potential beneficiaries through public transportation.

⁴ Ciudad Mujer administrative data.

CM catchment areas, and all services at both locations were provided free of charge. A third group of women was randomized to control and received no encouragement.

The evaluation sample was drawn through a multi-stage sampling procedure. The first stage selected a random sample of 567 out of 2,212 census segments (primary sampling units or PSUs) in the 19 municipalities in the catchment area of the Ciudad Mujer centers. Using updated cartography for the 567 PSUs, a random sample of 20 occupied households was selected with equal probability in each PSU and an updated listing of household members was collected to identify households meeting the inclusion criteria, namely having at least one female resident between 18 and 60 years old. From the list of households meeting the inclusion criteria, a random sample of 11 households were selected with probability proportional to the number of women in the household. Nine of these households were randomly selected for the original sample, and 2 were replacements in case an originally selected household was unavailable at the time of the survey. Finally, to avoid having women in different treatment groups within the same household, one woman between 18 to 60 years old was selected with equal probability in each sample household to be part of the evaluation sample, for a total of 4,618 women in the same number of households.

Randomization of the evaluation sample was implemented at the individual level. Each woman in the sample was randomly assigned to one of three groups (1) Ciudad Mujer (2) Placebo (health center) or (3) control with equal probability (1/3). To minimize the potential for treatment contamination, enumerators were also randomly assigned to one of these three groups, and trained to implement the corresponding encouragement protocol at the end of the baseline survey. Enumerators were provided with the household listings of households assigned to their group in their assigned PSUs, and detailed field procedures, supervision and quality assurance measures

ensured that the randomization protocol was strictly adhered to in the field. Given the wide dispersion of the sample across more than 2,000 PSUs in 19 municipalities, we argue that the potential for informational spillovers between the three groups is minimal. If treatment contamination or spillovers were to have occurred, this would likely involve women promoted to Ciudad Mujer informing the Placebo or control groups about the existence of Ciudad Mujer, which would increase participation in the non-intervention groups and attenuate our estimated impacts.

To design the encouragement strategy, the program hired a social marketing specialist to conduct formative research and to identify an encouragement mechanism that would increase program participation while not directly affecting the final outcomes. The proposed encouragement was a voucher to be exchanged at the CM or local health center for a \$15 gift card (equivalent to approximately one day's wages for salaried female workers in our sample),⁵ redeemable at a national supermarket chain. The voucher was non-transferable, so it could only be claimed by the woman participating in the experiment (this was enforced by noting the woman's name and ID number on the voucher) within a 30-day period. Only one voucher was issued per woman at the end of the baseline survey, and once the voucher had been claimed at the local CM or health center, no additional promotion or encouragement activities were implemented.

We hypothesized that the initial exposure to CM would serve to reduce information asymmetries regarding the existence of CM services, how to access them and the quality and potential benefits from utilizing CM services. All utilization of services and related outcomes after the initial encouraged visit would then be attributable as effects of the CM model, independent of the initial one-time encouragement.

⁵ Baseline survey descriptive statistics indicate an hourly average salary of \$1.72 for women in the study sample.

A second experimental group was randomly encouraged to visit a local health center, which serves as a proxy for existing service provisions. This placebo group allows us to indirectly test the exclusion restriction; that is, the observed effects of CM are generated through participation in CM and not indirectly through encouragement. For example, we might be concerned about the income effect of the \$15 supermarket voucher, which might free resources to cover the transportation costs to local social services. Furthermore, the placebo encouragement provides an approximate counterfactual estimate of service utilization under the existing fragmented model. While the placebo encouragement could be interpreted as a treatment on its own right, the experimental design was intended to evaluate the impact of CM relative to a fragmented service, rather than to evaluate the impact of visiting a local health center.⁶ We explore this question further in section 7.

The first stage of the instrument can be tested empirically by comparing participation in CM among the CM encouragement group relative to the placebo encouragement (HU) and a randomly assigned control group with no encouragement at all (NP). Figure 1 presents the proportion of first visits to CM in each treatment group over time according to CM administrative records. We can observe that the encouragement resulted in almost a 50-percentage point increase in initial visits, well above the 10% of women who visit the CM centers based on spontaneous demand. We argue that effects on social service utilization and related outcomes that take place after the first (encouraged) contact with CM are attributable to CM, independent of the initial encouragement, thanks to lowering initial informational asymmetries regarding the center.

⁶ Consistent with this interpretation, we find no evidence that the \$15 voucher increased health unit visits: the ITT effect of HU promotion on self-reported health unit attendance, estimated using only the HU and NP groups, is -0.007 ($SE = 0.023$), against a control-group mean of 0.797, leaving little scope for a voucher-driven increase. This outcome is not from administrative records, unlike CM visits, health unit attendance was not tracked administratively, and instead comes from self-reported answers in the baseline and endline surveys.

5. Data

To design the evaluation and the encouragement design, we took advantage of the timing of launch of three new CM centers. At the moment of the evaluation, only one center in the municipality of El Salvador was fully operational, and three new centers were about to be launched in the municipalities of San Martín, Santa Ana, and Usulután. The evaluation sample was constructed to be representative of Salvadorian women aged 18 to 60 living in the 19 municipalities contiguous to the three centers that were to be launched, resulting in a total sample of 4,618 women. The government estimated that the population of women located in these 19 municipalities constituted the potential beneficiaries of the program.

We used two types of data in our analyses, administrative data from CM centers and survey data from purpose-specific baseline and follow-up surveys. Baseline data were collected before the launch of the three new CM centers, between January and June 2013. The fieldwork followed the inauguration schedule of each CM center to minimize the chances of recording data from women who had already visited the facilities. Due to security conditions around the San Martín CM center, field teams were unable to interview 503 women in their homes. A total of 150 women were randomly selected for a subsample to be interviewed in shopping centers close to their communities. A sample of 112 women attended the interview. To ensure respondent privacy in this subsample, each woman was contacted in advance and given a specific appointment time, which ensured that only the respondent and the trained female enumerator were present during the interview. Interviews were conducted in areas with minimal pedestrian flow—either in discreet spaces outside the main commercial complex or inside the facility in designated seating areas away from public activity. These arrangements prevented the presence of bystanders, reduced the risk

of being overheard, and preserved the confidentiality and comfort of the respondents. As a result, the privacy standards for this subsample remained consistent with those applied in household interviews. We re-weighted this subsample to compensate for the sample attrition in the San Martin area. The effective sample consisted of 4,062 women.⁷

A follow-up panel survey was conducted between February and March 2014, between 8 and 15 months after the inauguration of the centers, following the sample of 4,062 women interviewed at baseline. A total of 216 women were unreachable during the follow-up data collection due to foreign migration, death, or incarceration, representing an attrition rate of 5.3%. Attrition was balanced between the randomized promotion and control groups (see Appendix Table A1). The final balanced panel used for analysis consists of 3,846 women with baseline and follow-up data.

The household questionnaire included detailed demographic and socioeconomic information about the sample of women and their households. Each woman was asked about her use of public services related to sexual and reproductive health, employment, and gender-based violence in the 12 months prior to the interview. Additionally, women were asked to rate their subjective well-being.

Administrative data captured women's attendance at the centers, the type of services women were referred to after receiving guidance on their first visit to CM centers, and the services/modules they actually received/visited. These data were then merged with the survey data. The administrative database included data from January 2013 to May 2014, providing around a year for women to attend CM.

⁷ Of the total 4,062 women in the sample, 2,707 were assigned to one of the two encouragement groups. A total of 2,626 accepted the voucher, and 71 women rejected the voucher. The encouragement groups were incorrectly assigned for 35 women. These women maintain their original random assignment status for purposes of the analysis.

For the analyses, we used the CM centers' administrative data to quantify the effectiveness of our encouragement to visit the CM centers and the survey data to measure the utilization of services in the CM centers and in the health centers/units.

The primary outcome variables are related to the use of public services. We constructed both an aggregate index that comprised a total of 20 specialized services on sexual and reproductive health, economic autonomy and gender-based violence support, as well as five sub-indices grouped by the type of service: i) sexual and reproductive health care; ii) promotion of female employment; iii) psychological, medical and legal support for survivors of physical, sexual and/or emotional violence; iv) legal support for victims of patrimonial violence; and v) legal services to strengthen economic autonomy (see Table 1 for a detailed description of services).

The indices express the proportion of public services used by each woman during a 12-month reference period, independent of where the service took place. The indices are constructed as follows:

$$Index_i = \frac{1}{N} \sum_{j=1}^N s_{ji},$$

where N is the total number of available services, and s_{ji} represents the service j used by women i , which takes the value of 1 if the woman used the service in the reference period and zero otherwise.

To interpret the magnitude of change in standard deviations, we convert indices to z-scores:

$$z-Index_i = \frac{(Index_i - \overline{Index_c})}{SD_c},$$

where $\overline{Index_c}$ and SD_c are the mean and standard deviation in the control group, respectively (Kling et al., 2007).

6. Identification Strategy

As discussed in sections 2 and 4, CM is open for all women in El Salvador, and services are provided for free. As such, the program model precludes the option of restricting access for evaluation purposes. We estimate the impact of CM on public service utilization by leveraging the experimental variation generated by the random encouragement for an initial CM visit (extensive margin). Specifically, we use the treatment (promotion) assignment as an instrument for program participation to estimate the treatment on the treated (TOT) for compliers.⁸ We specify the two-stage least squares (2SLS) analysis separately for CM compared to HU promoted women, and CM compared with control NP women. Using the traditional IV approach in the randomized controlled trials (RCT) literature and controlling for the dependent variable in the baseline period to increase precision, the specification is as follows:

$$(1) \quad VisitedCM_i = \alpha_0 + \alpha_1 PromotionCM_i + \alpha_2 y_i^{BL} + \alpha_3 X_i^{BL} + \mu_i,$$

$$(2) \quad y_i = \delta_0 + \delta_1 \widehat{VisitedCM}_i + \delta_2 y_i^{BL} + \delta_3 X_i^{BL} + \varepsilon_i,$$

where y_i is the outcome of interest for woman i . $VisitedCM_i$ and $PromotionCM_i$ are indicator variables for whether woman i has ever visited a CM center and whether she was encouraged/promoted to visit CM, respectively. Furthermore, y_i^{BL} is the outcome variable in the baseline period for woman i , and X_i^{BL} are baseline characteristics of woman i such as age, head of household, has a couple, couple lives in the household, number of rooms in household, number of children, number of children younger than 5 living in household, literate, years of education,

⁸ Compliers are women who attend a CM center if promoted.

socioeconomic index⁹, owner of house, employed, labor income, and affiliated to a public or private social security system. Finally, μ_i and ε_i denote idiosyncratic random errors. We refer to this specification as ‘Cross-section’ estimates in our result tables because, as explained below, we also report results considering the two periods comprising our data.

Following Solon et al. (2015), we reweighted the observations in our estimates corresponding to equations (1) and (2) by the inverse probability of being selected in the sample. This endogenous sampling correction acknowledges that, as described in section 5, only a randomly selected 150 sub-sample out of the 503 women in the San Martin area could be reached to be surveyed in a safe place near their communities.

The first stage of our 2SLS specification, equation (1), predicts the probability of visiting a CM center according to the randomized promotion group assigned to each individual. CM visits records are obtained from administrative databases of the corresponding CM centers. The second stage, equation (2), estimates the impact of having visited CM on subsequent outcomes (intensive margin), denoted by y_i . The second-stage outcome variables need to capture the subsequent use of services not only in CM centers, but also in health units/centers. For this, we used baseline and follow-up surveys information comprising the three encouragement groups. The coefficient δ_1 in equation (2) is the parameter of interest and represents the TOT of CM for compliers.

With the data collected in the baseline survey, we tested if the characteristics of the treatment, placebo and control groups were, on average, not statistically different after the randomization

⁹ For the socioeconomic index, higher (lower) values represent worse (better) living conditions. The variable is the sum of the following dichotomous variables: *Has a substandard housing (it is not a private or independent house, apartment or condominium)*; *Materials of the walls are precarious (it is not concrete, brick, or wood)*; *Housing has no water*; *Housing has no electricity*; *Housing has no private toilet-flush water*.

process¹⁰. Tables 2 and 3 show balance tests between the encouraged groups for a set of baseline sociodemographic characteristics and for our 5 outcome variables of interest, respectively. While in most cases the magnitude of the differences is small (or explained by very low baseline levels), our analysis indicates a proportion of baseline unbalanced variables between groups that is higher than expected based on chance.¹¹ Specifically, the baseline sociodemographic characteristics of the women promoted to CM are balanced with the group of women assigned to no promotion (NP). Nevertheless, the CM group has, on average, a higher probability of being married and living with her partner in the household, less likely to be literate or being employed, and live in worse conditions, relative to women encouraged to visit a health unit (HU). Regarding the outcome variables, women promoted to visit a CM center have previously used fewer services than those promoted to HU or NP groups.

To address this imbalance, we control for baseline differences and all other non-time varying observable and unobservable characteristics using a fixed-effects (FE) specification. Thus, our preferred empirical identification strategy exploits both the exogenous temporal variation (the baseline survey at time t_0 and the follow-up survey at time t_1) and the exogenous variation in the assigned encouragement. Therefore, in this approach, the variation used for identification comes from differences in our indicators of interest over time within individuals, and between the treatment and comparison groups. The implicit assumption is that unobservable differences in the

¹⁰ It is expected that, on average, one out of ten variables compared between the groups can be unbalanced at a 90% confidence level (Glennester and Takavarasha, 2013), and when more than that occurs, it could also be a random event, even if the random allocation of women to the different groups was properly implemented.

¹¹ As a reference, we also included some descriptive statistics for women at the national level based on the 2013 Multipurpose Household Survey. The characteristics of women in the study are, in general, similar to those in the whole country.

absence of the program between groups would have had similar effects on the indicator of interest over time. The specification in each stage is described as follows:¹²

$$(3) \quad VisitedCM_{it} = \beta_0 + \beta_1 PromotionCM_{it} + \beta_2 post_t + \theta_i + \epsilon_{it},$$

$$(4) \quad y_{it} = \gamma_0 + \gamma_1 \widehat{VisitedCM}_{it} + \gamma_2 post_t + \rho_i + v_{it},$$

where y_{it} is the outcome indicator of interest for woman i in period t . The indicator variable $post_t$ establishes whether the observation is from the second period (follow-up), controlling for the existence of shocks that may have affected all women over time, and θ_i and ρ_i denote individual-level fixed effects that capture the time invariant observable and unobservable characteristics of woman i . Furthermore, $PromotionCM_{it}$ and $VisitedCM_{it}$ are dichotomous variables that take value 1 *in the second period* if the woman i was promoted or has visited CM, respectively, and 0 otherwise.¹³ Therefore, β_1 is the effect of the incentive to visit a CM center on the probability of visiting one of these three centers, and γ_1 is our parameter of interest and represents the TOT of visiting a CM center on subsequent outcomes (intensive margin). Finally, ϵ_{it} and v_{it} denote idiosyncratic random errors. For all IV-FE estimates we report second-stage result tables which include the Anderson-Rubin test p-value for weak-instrument-robust inference.

Additionally, we estimate intention-to-treat effects (ITT) related to the promotion using the full sample and the FE model. Importantly, this exercise allows us to understand whether the estimated effects of our main model are a result of the encouragement itself, the \$15 monetary

¹² This specification is analogous to equation (2) but using the change in the outcome variable, Δy_i (i.e., $y_{it} = 1 - y_{it} = 0$), as the dependent variable.

¹³ In other words, $PromotionCM_{it}$ is an indicator for the post-treatment period for individuals who received the treatment (encouragement to visit CM).

incentive present in both the HU and CM promotion groups, or the CM service delivery model. Specifically, if the two promotions have significant effects on the estimated outcome, it suggests that the results are, at least in part, due to the provided incentive.

The estimated ITT specification is as follows:

$$(5) \quad y_{it} = \pi_0 + \pi_1 PromotionHU_{it} + \pi_2 PromotionCM_{it} + \pi_3 post_t + \varphi_i + v_{it} ,$$

the parameters π_1 and π_2 capture the ITT effects of promotion to HU and promotion to CM, respectively. The results are presented in the Appendix (Tables A2 and A3) and are largely consistent with our main estimates.

To correct for multiple hypotheses, we implement a stepwise multiple testing method, proposed by Romano and Wolf (2005), in all our main estimates. Considering the multiplicity of tests (i.e., the number of dependent variables), the probability that a certain hypothesis is rejected by pure chance may be too high. To tackle this concern, we control using the familywise error rate, which is the probability of having one or more false discoveries, to ascertain (at the confidence level chosen) that the results found are not simply due to chance.

Finally, to understand potential mechanisms at play, we also explored heterogeneous treatment effects by baseline characteristics. We examine differential effects by socioeconomic status, education, age, and the number of children (between 0 to 12 years of age) in the household.¹⁴ The estimated IV second-stage equation corresponds to:

$$(6) \quad y_{it} = \lambda_0 + \lambda_1 \widehat{VisitedCM}_{it} \cdot H_i + \lambda_2 \widehat{VisitedCM}_{it} + \lambda_3 H_i \cdot post_t + \lambda_4 post_t + \eta_i + v_{it},$$

¹⁴ We considered the 0-12 age range because it corresponds to the group of children covered by the CM centers' on-site childcare.

where H_i represents the heterogeneity variable (socioeconomic index, years of education, age, and the number of children in the household) at the baseline period. In this case, the heterogeneous differential effect is captured by λ_1 .

We also report ITT heterogeneous effects using the same set of heterogeneity variables and a specification analogous to equation (5). All our estimates include standard errors clustered at the primary sampling unit (PSU) level.

7. Results

7.1. Main Findings

Our main finding is that the provision of integrated services through CM leads to an increased demand for services among compliers. Results in Table 4 show the extensive margin effect of the treatment (encouragement to visit CM) on the likelihood of visiting a CM center. There is a large and highly significant first stage, with almost a 50-percentage point increase in the probability of visiting at least once a CM center, relative to both the HU and NP groups, respectively. Additionally, the overall take-up was 63% for the group promoted to CM, while 84% of women promoted to visit an HU did so at least once. The higher take-up level in the HU group might be explained by a greater preference for visiting an already known place and by health units/centers being more accessible in geographical terms.¹⁵ Importantly, the large first-stage F-statistic shows that our instrument is highly correlated with the endogenous variable and allows us to reject the null hypothesis of weak instruments.

¹⁵ The number of health units in the CM influence area is 9, compared to the 3 CM centers.

Table 5 shows a substantial impact of CM on the intensive margin of visits, and the demand for additional (beyond the induced first health visit) public services, as shown in Columns (I) and (II). In addition to overall subsequent visits, we also examine subsequent non-reproductive health visits. Column (III) reports the impact on the probability of having at least one such non-reproductive health follow-up visit, and Column (IV) reports the effect on the number of these visits. We find that women who visited CM had a 68-percentage point increase in the probability of returning to the centers at least once, with about half of these follow-up visits related to non-reproductive health services. On average, they accumulated approximately 2.1 additional subsequent visits, of which about 0.6–0.7 were for non-reproductive health services. Overall, we found few differences in significance levels, magnitude, or sign of estimated impacts when comparing the CM group to either the placebo (HU) or the non-promoted (NP) groups.

Table 6 reports the impacts of CM on the utilization of public services using the aggregate index of 20 public services, along with sub-indices by type of services offered, and life satisfaction. The results show that CM increased the use of public services by 0.53 standard deviations compared to the non-promoted group, with significant and large effects in the realm of reproductive and sexual health, and legal services to support victims of patrimonial violence or to strengthen economic autonomy. More specifically, CM increased the use of reproductive and health services by 0.48 standard deviations compared to the pure control group. The use of legal services to deal with patrimonial violence or to strengthen economic autonomy increased by 0.29 and 0.35 standard deviations, respectively, compared to the control group.

Additionally, we find positive effects on the use of services to promote female employment; however, these are statistically significant only when comparing the CM group to the HU group.

This may reflect CM's stronger emphasis on integrated support for labor force participation relative to the existing, fragmented service model.

On the other hand, we did not find clear evidence in the short run (i.e., 8-15 months after the inauguration of the centers) of a significant increase in the demand for services that support survivors of physical, sexual and/or emotional violence. After the establishment of CM, the center might require time to reach its full potential, and there might be a need to further strengthen some processes and services during the program's initial stages. In either case, the exploration of longer-term impacts is an important avenue for future research.

In addition, women who visited CM were between 8 and 10 percentage points more likely to report being satisfied or very satisfied with their life, which is a relative increase of between 10% and 13% over the comparison groups.

Results linked to the placebo control group that received encouragement to visit the local health clinic under the fragmented service delivery model (HU) yield similar but generally larger impacts than the ones described to the pure control group (NP). These differences may be driven by a dissimilar composition of the compliers' groups. To better understand how compliers might differ when comparing to the placebo or pure control groups, we need to recall that the promotion to CM might have two components that can potentially remove the constraints to comply. The first one is related to the voucher itself, which can compensate for the opportunity cost of going to a center. The second one is information about CM centers (e.g., location). Women in the pure control group (NP) did not receive the voucher, nor the information. On the other hand, women promoted to HU get the voucher and could also receive (not as part of the promotion itself) some information about CM when visiting a local health unit. If that is the case, a group of women who face an information constraint and are promoted to HU might be considered always-takers, while the same group

wouldn't be considered as always-takers if not promoted (NP). As the proportion of never-takers is not affected, the result is a different composition of compliers that might explain the observed larger impacts when comparing CM to HU. More details about compliers, always-takers and never-takers are presented in subsection 7.3.

Finally, looking at the ITT results, we only found statistically significant effects on the demand for services (Table A2) or the use of them (Table A3) for the CM promotion but not for the HU promotion. This suggests that the CM service delivery model, and not the US\$ 15 encouragement, caused the observed effects.¹⁶

7.2. Impacts on Specific Services

As discussed in our general findings in section 7.1, the increase in the use of the aggregate level of services caused by CM is explained by higher use of sexual and reproductive health services, legal services for victims of patrimonial violence, and legal services to strengthen women's economic autonomy. To better understand which specific services boosted the aggregate use of services, we examined the isolated short-term impact of CM on the utilization of each public service considered in these three services categories.

In the sexual and reproductive health domain, two preventive services show the clearest gains. Women encouraged to visit Ciudad Mujer were 26 percentage points more likely to receive a Pap smear in the past year, and 14 percentage points more likely to get a mammogram, compared to those not encouraged (Appendix Table A4). These are large effects in relative terms: Pap test

¹⁶ To directly address whether the HU encouragement functioned as a treatment in its own right, we estimated the ITT effects of HU promotion using only the HU and NP groups, following the same fixed-effects specification. We find no significant effects of HU promotion on any of the service use indices or life satisfaction outcomes in Table 6, with Romano-Wolf corrected p-values of 0.40 or above across all outcomes. These results confirm that the effects of CM are driven by the integrated service delivery model, not by the voucher.

utilization increased from 56% in the control group to 82% in the treatment group (a 46% increase), while mammogram uptake rose from 7% to 21%, representing a 200% increase. By contrast, no significant improvements were observed for services related to pregnancy, such as prenatal or postnatal check-ups. These null results should be interpreted cautiously, since the sample was not designed to assess impacts specifically among pregnant women. Overall, the increased utilization of Pap smears and mammograms appears to be a key driver of the broader reproductive health index gains, aligning with Ciudad Mujer's focus on gynecological preventive care.

Regarding legal services for victims of patrimonial violence, the most substantial effects are seen in assistance with paternity acknowledgment and with legalizing property or assets (Appendix Table A5). Women who visited Ciudad Mujer were 5 percentage points more likely to receive support in establishing paternity for their children—a more than threefold increase over the 1.2% baseline rate. A similar effect size is observed for legal support related to property or asset formalization. However, these legal service effects are measured on very low base rates and, while significant relative to the health-unit placebo group, they are not statistically significant compared to the pure control group once we account for multiple-hypothesis testing. Thus, while these findings suggest that Ciudad Mujer's one-stop model helped some women address patrimonial legal issues (like child support and property rights), the evidence is tentative in the short run.

Finally, services aimed at strengthening economic autonomy show clear positive impacts, particularly in helping women secure personal identification documents. Ciudad Mujer's integrated centers significantly increased women's access to civil documentation services, as reflected in higher take-up of government ID cards and birth certificates (Appendix Table A6). Women encouraged to use Ciudad Mujer were 7–11 percentage points more likely to obtain a national ID or their birth certificate within the year. In practical terms, the share of women getting assistance

for an ID rose from about 4% in the control group to roughly 11% in the treatment group, and for birth certificates from about 3% to 7.6% in the treatment group. These increases (over 2.5 times the baseline rates) are statistically significant and underscore Ciudad Mujer's effectiveness in reducing bureaucratic barriers for women. Having an official ID or birth certificate is crucial for economic inclusion (enabling activities like opening bank accounts or accessing credit), so these gains may have downstream benefits for women's autonomy.

In contrast, and consistent with results from the aggregate indices, we do not find significant short-term increases in the use of specific services related to female employment support or services for survivors of physical, sexual, or emotional violence (Appendix Tables A7 and A8). These null results may reflect limited implementation maturity in these service areas during the early stages of the program or a need for more targeted outreach.

In summary, the main drivers of Ciudad Mujer's aggregate service impact are a higher uptake of critical health screenings (Pap smears and mammograms) and improved access to legal and documentation services (especially for civil identification and certain patrimonial rights), with other services showing more limited short-run effects. These findings help explain the observed gains in life satisfaction and reinforce the importance of integrated service delivery in improving access to critical but underutilized services.

7.3. Compliers Analysis

To better understand the subgroup of women we are making inferences about and properly interpret our TOT estimates, we characterized the groups of compliers and non-compliers in our IV analysis. Following the decomposition by Marbach and Hangartner (2020), Table 7 shows group means and differences of some sociodemographic variables for compliers, never-takers, and

always-takers. In our sample, approximately half of the women would attend a CM center if promoted to go (compliers). Around 15% would go regardless of receiving the incentive or not (always-takers), and 35% wouldn't attend even if promoted to go (never-takers).

The decomposition also reveals a larger group of always-takers and a smaller group of compliers when comparing the CM promotion to the HU promotion, relative to the comparison with the non-promotion (NP) group. As explained in subsection 7.1, differences in the composition of compliers across these groups might help explain why treatment effects are larger in magnitude when comparing CM to HU. In particular, point estimates suggest that compliers in the NP group are, on average, older, have more children, are less educated, and live in worse conditions than compliers in the HU group. These characteristics may constrain their ability to use services, potentially explaining the smaller effects observed in the CM vs NP comparison.

Regarding sociodemographic characteristics, complier women are, on average, closer to always-takers than to never-takers. Compliers are, on average, two to three years older, are less educated compared to non-compliers, and have more children. Having more children is particularly relevant because it might indicate the relevance of CM centers' childcare to attract women. Also, compliers are more likely to be affiliated to a social security system and to live in worse conditions than never-takers.

In a similar analysis, Table 8 reports differences for services' use indices at baseline between compliers and non-compliers. Overall, during the 12 months before the baseline survey, compliers used fewer services than always-takers but used more services relative to never-takers.

The largest difference between the three groups can be observed in the use of services for survivors of physical, sexual, or emotional violence. One reason that could explain the uneven use of these services is a different level of need to use those services between groups. To test that, we

analyzed the balance in women's baseline self-reported level of intimate partner violence (results not shown), and we found no statistically significant differences between the three groups.

In this way, there is a small group of women (15%) that have been using more services relative to the rest and would attend a CM center even without being encouraged (always-takers). The reason might be because they face fewer information constraints and the expected return of attending a CM center is higher than their alternative cost. Always-takers might face fewer alternative costs than compliers because, on average, they have fewer children, and therefore do not need daycare or might already have someone to leave them with. There is a second group of women (35%) who won't go to CM if promoted (never-takers) and are different from compliers and always-takers. They live in better conditions, have more education, and previously used fewer services. With higher opportunity costs, never-takers might not find the value of the voucher or the free services provided in the CM centers attractive enough. These possible explanations gain strength when we observe that never-takers have a statistically significant higher probability of being covered by a health insurance (results not shown) and have fewer children than compliers, making the need for childcare a non-binding restriction.

Finally, 50% of women for whom we estimate effects when interpreting the TOT are compliers, i.e. those who only attend a CM center if encouraged. They are, on average, older, less educated than non-compliers, and importantly, have more children. Compliers could face a combination of information constraints (not knowing where to access these types of services) and a higher alternative cost of attending any type of centers. Thus, the promotion may have removed the information restriction, and similarly, the voucher value combined with childcare availability within the CM centers may have generated a positive net balance between the costs and benefits of visiting a CM center.

As a result, we conclude that the CM model is more attractive for the most vulnerable group of women and those who did not have childcare available. These findings hold when considering the placebo or the control group as comparison in the IV model.

7.4. Additional Findings

To better understand how Ciudad Mujer may be leading to increased utilization of services, we conducted additional analyses of impacts on our main outcomes by women's baseline socioeconomic condition, years of education, age, and the number of children below 12 years of age living in the household. Instrumental Variables (TOT) heterogeneity effect estimates are presented in Tables 9 and 10 for all the main outcomes.

When we examine heterogeneous effects for the number and the probability of having subsequent visits to CM, the TOT effects reported in Table 9 are significant for years of education, but not for socioeconomic status, age, or the number of children in the household. Women with more years of education were less likely to have a subsequent visit to a CM center and had fewer visits to the centers compared to women in the control and placebo groups. Also, despite not being statistically significant, point estimates suggest a greater propensity to visit a CM center for those women with a higher socioeconomic index (i.e., living in worse conditions).

Similarly, in Table 10, we observe that the estimated TOT on service utilization increases significantly with women's age. This heterogeneous effect by age on the aggregate index of public services is explained, as shown in column I.1, by a larger use of reproductive and health services by older women. We did not find statistically significant differentiated effects by socioeconomic index, the years of education, or the number of children in the household. Nevertheless, point

estimates suggest a lower use of services by women who live in worse conditions and have more children below 12 years of age in the household.

In a similar exercise, we report ITT heterogeneous estimates on the demand and the use of public services in the Appendix Tables A9 and A10, respectively. As expected, the estimated effects mirror the heterogeneity results found for TOT. The statistically significant and positive effect of the promotion to CM on the probability and number of visits to CM decreases in magnitude with the women's years of education. There also appears to be a positive differentiated effect of the promotion on the likelihood of going to a CM center by age. Regarding the use of services, again, we only found a statistically significant heterogeneous effect of the promotion to CM by age, which is also explained by higher use of sexual and reproductive health services by older women.

We additionally present the statistically significant heterogeneous ITT estimates graphically (Figures 2 and 3). Specifically, we show linear fit lines for each of the three promotion groups. The y-axis shows the individual difference between the second- and first-period outputs, while in the x-axis is the heterogeneity variable. Therefore, the vertical differences between two lines represent the corresponding FE ITT estimator through all values of the heterogeneity variable. For example, Figure 2 shows significant positive differences between women promoted to CM and both the placebo and the control groups on the probability of having a subsequent visit to CM throughout all levels of education. The CM fitted line has a negative slope, so the difference (i.e., the ITT) becomes smaller as years of education increase. On the other hand, Figure 3 shows an analogous relationship between the change on aggregate use of services and women's age. The estimated ITT effects are not statistically different to zero for women below 30 years of age, while it becomes statistically significant and increasing with age for women older than 30.

These results are consistent with the CM model's objective of making services more accessible to the more vulnerable segments of the population who face significant barriers to accessing services, whether those were the cost of time, transport, opportunity cost of childcare or lack of knowledge, information and experience with social services. Overall, we conclude that CM centers have had larger impacts on the service demand and utilization for what might be characterized as the more vulnerable groups of less educated and older women.

Finally, even though our data cover only one year of operation of the CM centers, we estimated the impact of CM on a set of women's outcomes that could be affected by using the public services considered in this study. For example, Appendix Tables A11 and A12¹⁷ show estimated results for the use of contraception methods and being on treatment for a group of common health conditions like diabetes, respectively. Also, Appendix Tables A13 to A15 show estimations for labor outcomes, having experienced intimate partner violence, and female empowerment. We do not find significant effects on any of the variables included in the analysis. It is important to contextualize these results within the limited time elapsed between the opening of the three CM centers and the follow-up survey.

8. Conclusion

This study represents the first rigorous evaluation of an OSS model specifically designed to cater to women's needs in a developing country context. By employing a randomized encouragement design, we assessed the impact of CM on service utilization and subjective well-being among women in El Salvador. Our findings reveal that the integrated services provided

¹⁷ This table shows the cross-section specification results because the health conditions were asked in the follow-up survey but not in the baseline survey.

through CM centers significantly increase the use of public services, particularly in areas related to sexual and reproductive health care, legal support for victims of patrimonial violence, and services aimed at strengthening economic autonomy.

Approximately one year post-intervention, women encouraged to visit CM centers showed a 10% increase in self-reported well-being and a 0.53 standard deviation increase in service use compared to control groups. The most substantial effects were observed in reproductive and sexual health services (0.48 SD), legal services for victims of patrimonial violence (0.29 SD), and legal services to strengthen economic autonomy (0.35 SD). The CM model proved particularly effective for older, less educated, and lower-income women, indicating its potential to target the most vulnerable populations. However, no significant short-term effects were found on female employment services or support for survivors of physical, sexual, and emotional violence, suggesting these areas may require more time to show measurable impacts or further programmatic adjustments.

Evaluating the short-term impact of the CM approach is crucial as it provides initial insights into the effectiveness of integrating services to address women's multiple needs. From a policy perspective, this approach introduces several crucial elements to the provision of public services for women. First, it reduces the economic opportunity cost for beneficiaries by concentrating multiple services in one location, thereby saving time and money. Second, the integrated woman-focused approach allows for coordinated and customized service delivery, enhancing inter-institutional coordination and service quality. These insights are essential for making informed decisions about further investments and adaptations of the CM model in different regions and contexts.

Several plausible mechanisms, grounded in the program's design, could explain how the Ciudad Mujer model enhances subjective well-being. A first mechanism is the reduction in transaction costs and convenience barriers for women seeking help. By consolidating multiple services in one location, the one-stop centers sharply cut the time, travel, and financial burden required to access support. In our context of El Salvador, where women often faced long trips and wait times to reach disparate agencies, the ability to address health, legal, and economic needs under one roof would save significant time and money. This efficiency likely reduces stress and forgone income, thereby improving overall life satisfaction. Indeed, the one-stop model was explicitly designed to minimize bureaucratic hurdles – for example, Ciudad Mujer uses a one-time registration and queue system that streamlines service delivery, eliminating the need for multiple enrollments or repeated visits. By lowering these logistical barriers, the program may free up women's time for family or work and diminish the frustration associated with navigating public services, leading women to feel more content with their lives.

A second, related mechanism is the enhanced quality and integration of services, which can have direct emotional and psychological benefits. Ciudad Mujer's centers provide a safe, women-centered environment: all staff are female and trained in a gender-sensitive, rights-based approach, and only women are admitted as clients. This design likely fosters a sense of security and trust. Women may feel more comfortable discussing sensitive health or family issues when the setting is explicitly supportive and tailored to them. The availability of on-site childcare is another critical feature – mothers can attend to their needs knowing their children are safe nearby, which reduces worry and encourages uptake of services.

Moreover, Ciudad Mujer employs an integrated case management and personalized counseling model. Upon a woman's arrival, a professional counselor conducts an intake, listens to her concerns

across various life domains (health, employment, home safety, etc.), and then provides individualized guidance by creating a roadmap of services to address her specific needs. This kind of personalized attention and holistic support can directly improve a beneficiary's emotional well-being: women feel heard, respected, and cared for rather than being lost in a bureaucratic system. The counselor's guidance ensures that women get the right mix of services for their situation, potentially yielding quicker or more tangible improvements in their lives (for example, simultaneously addressing a health issue and a legal concern). In sum, the combination of reduced barriers, higher perceived safety, and empathetic, coordinated care under the Ciudad Mujer model can plausibly translate into greater life satisfaction. Women not only access more services, but they do so in a dignified manner – which may empower them and improve their outlook on life, even within the relatively short horizon of our study.

Our study contributes to the literature on gender equality policies by using an experimental approach to obtain causal impacts of the CM program. Overall, the findings suggest that the OSS model effectively boosts the demand for essential services among women. These results provide valuable insights for policymakers aiming to improve the efficiency and equity of public service delivery, emphasizing the importance of addressing barriers to access and tailoring services to the needs of women facing structural disadvantages. However, further research is needed to explore the longer-term impacts of this approach. Future studies should focus on understanding the effects of OSS models on female employment and support for survivors of physical, sexual, and emotional violence, as well as the sustained benefits of integrated service delivery.

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TABLES

Table 1. Public Services Considered in the Aggregate Service Index, by Area

Sexual and reproductive health services	▪ Had a cytology/Pap in the last 12 months ▪ Had a mammography in the last 12 months ▪ Had a prenatal checkup for women who have an actual pregnancy/have given birth in the last 12 months ▪ Had a dental checkup for women who have an actual pregnancy/ have given birth in the last 12 months ▪ Had a postnatal checkup for women who have given birth in the last 12 months
Services to promote female employment	▪ Requested or received job training sessions in the last 12 months ▪ Requested or received job placement services (registration in job opportunities, assistance to prepare CV or help for a job interview) in the last 12 months ▪ Requested or received orientation to start or improve their own business in the last 12 months ▪ Requested or received credit/monetary support to open or expand a business in the last 12 months
Psychological, medical and legal services for survivors of physical, sexual and/or emotional violence	▪ Sought/received emotional support in the last 12 months ▪ Sought/received legal support in the last 12 months ▪ Sought/received injunction for protection in the last 12 months ▪ Sought/received medical aid in the last 12 months ▪ Sought/received transportation support in the last 12 months ▪ Sought/received support to file a complaint in the last 12 months
Legal services for victims of patrimonial violence	▪ Received help for the acknowledgment of the paternity of their children in the last 12 months ▪ Received help for food fees in the last 12 months ▪ Received help for legalization of property/assets in the last 12 months
Legal services to strengthen economic autonomy	▪ Received help to obtain her identification document (ID) in the last 12 months ▪ Received help to obtain her birth certificate in the last 12 months

Table 2. Balance of Women's Baseline Socioeconomic Characteristics

	Mean				Difference		Difference (%)	
	CM (N = 1,297)	HU (N = 1,282)	NP (N = 1,267)	EHPM (N = 1,978,692)	CM - HU	CM - NP	CM - HU	CM - NP
Age	36.693 (0.426)	36.417 (0.423)	36.762 (0.406)	35.627	0.276 (0.602)	-0.070 (0.569)	0.8	-0.2
Head household	0.258 (0.014)	0.269 (0.015)	0.258 (0.014)	0.219	-0.011 (0.020)	0.000 (0.020)	-4.0	0.1
Married or with a partner	0.609 (0.017)	0.545 (0.017)	0.573 (0.018)	0.521	0.064*** (0.023)	0.036 (0.025)	11.7	6.3
Partner lives in the household	0.589 (0.018)	0.524 (0.017)	0.552 (0.018)		0.065*** (0.024)	0.037 (0.024)	12.4	6.7
N alive born children	2.459 (0.078)	2.302 (0.075)	2.411 (0.067)		0.157 (0.108)	0.048 (0.098)	6.8	2.0
N children 0-5 years old living in the household	0.324 (0.017)	0.310 (0.018)	0.309 (0.018)		0.014 (0.024)	0.015 (0.025)	4.5	4.9
N children 6-15 years old living in the household	0.453 (0.022)	0.432 (0.021)	0.433 (0.023)		0.021 (0.030)	0.020 (0.030)	4.9	4.6
Literate	0.884 (0.011)	0.916 (0.010)	0.902 (0.010)	0.880	-0.032** (0.014)	-0.018 (0.015)	-3.5	-2.0
Years of education	7.872 (0.189)	7.858 (0.188)	7.673 (0.164)	7.869	0.014 (0.232)	0.199 (0.209)	0.2	2.6
Employed	0.577 (0.016)	0.624 (0.017)	0.569 (0.017)		-0.047** (0.023)	0.008 (0.022)	-7.5	1.4
Labor income (IHS)	3.297 (0.099)	3.465 (0.105)	3.219 (0.103)		-0.167 (0.144)	0.079 (0.137)	-4.8	2.4
Affiliated to a public or private social security system	0.158 (0.013)	0.167 (0.014)	0.157 (0.015)		-0.009 (0.020)	0.001 (0.019)	-5.6	0.4
Socioeconomic index	0.193 (0.009)	0.182 (0.009)	0.197 (0.010)	0.199	0.011 (0.008)	-0.004 (0.008)	5.9	-2.1
Household owner	0.598 (0.016)	0.638 (0.017)	0.625 (0.017)		-0.040* (0.022)	-0.027 (0.022)	-6.2	-4.3

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion. EHPM = Multipurpose Household Survey 2013.

Socioeconomic index is average of indicators: Has a substandard housing (it is not a private or independent house, apartment, or condominium); Materials of the walls are precarious (it is not concrete, brick, or wood); Housing has no water; Housing has no electricity; No private toilet-flush water.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. *** p<0.01, ** p<0.05, * p<0.1

Table 3. Balance of Baseline Outcome Variables

	Mean			Difference		Difference (%)	
	CM (N = 1,297)	HU (N = 1,282)	NP (N = 1,267)	CM - HU	CM - NP	CM - HU	CM - NP
Aggregate index of public services	0.051 (0.002)	0.056 (0.002)	0.060 (0.002)	-0.006** (0.003)	-0.010*** (0.003)	-10.0	-16.3
Index of sexual and reproductive health services	0.136 (0.005)	0.133 (0.005)	0.147 (0.006)	0.003 (0.007)	-0.012 (0.008)	2.3	-7.9
Index of services to promote female employment	0.021 (0.002)	0.027 (0.003)	0.023 (0.003)	-0.006* (0.003)	-0.002 (0.004)	-21.5	-7.5
Index of psychological, medical and legal services for survivors of physical, sexual, and/or emotional violence	0.019 (0.004)	0.018 (0.003)	0.019 (0.003)	0.001 (0.004)	0.000 (0.005)	7.3	1.9
Index of legal services for victims of patrimonial violence	0.021 (0.003)	0.030 (0.003)	0.029 (0.003)	-0.010** (0.005)	-0.008* (0.005)	-31.4	-28.5
Index of legal services to strengthen economic autonomy	0.036 (0.005)	0.077 (0.008)	0.090 (0.009)	-0.042*** (0.009)	-0.054*** (0.011)	-54.0	-60.4
Very Satisfied/Satisfied with life in general	0.801 (0.012)	0.826 (0.012)	0.824 (0.012)	-0.025 (0.017)	-0.024 (0.017)	-3.0	-2.9

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Index of sexual and reproductive health services: Prenatal checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Dental checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Postnatal checkup in the last 12 months, women who have given birth in last 12 months; Citology/Papanicolaou in the last 12 months; Mammography in the last 12 months, Women older than 40 years old.

Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence: Sought/received emotional support; Sought/received legal support; Sought/received injunction for protection; Sought/received medical aid; Sought/received transportation support; Sought/received support to put a complaint.

Index of legal services for victims of patrimonial violence: Has received help for acknowledgment of paternity of their children; Has received help for legalization of property/assets; Has received help for food fees.

Index of legal services to strengthen economic autonomy: Has received help to obtain her ID; Has received help to obtain her birth certificate.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. *** p<0.01, ** p<0.05, * p<0.1

Table 4. Instrumental Variables First-Stage

Dep. Variable: Visited CM				
	Fixed Effects		Cross-Section	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Promoted to CM	0.452*** (0.020)	0.469*** (0.020)	0.455*** (0.019)	0.474*** (0.020)
F-statistic	521.4	551.8	565.0	549.8
Partial R-squared	0.220	0.239	0.226	0.243
N	5,158	5,128	2,579	2,564

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

Cross-Section: Baseline covariates included; age, head of household, has a couple, couple lives in the household, N rooms in household, N children, N children < 5 living in household, literate, years of education, socioeconomic index, owner of house, employed, labor income, affiliated to a public or private social security system, and the dependent variable in baseline.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. *** p<0.01, ** p<0.05, * p<0.1

Table 5. Impacts on Demand

		Instrumental Variables Second Stage							
		(I) Probability to have a subsequent visit to CM		(II) Number of subsequent visits to CM		(III) Probability to have a non- reproductive health subsequent visit to CM		(IV) Number of non-reproductive health subsequent visits to CM	
		CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Fixed Effects	Visited CM	0.678*** (0.027) [0.001]	0.676*** (0.027) [0.001]	2.023*** (0.222) [0.001]	2.191*** (0.199) [0.001]	0.371*** (0.027) [0.001]	0.356*** (0.027) [0.001]	0.622*** (0.144) [0.001]	0.713*** (0.128) [0.001]
	Control Group's Mean	0.084	0.074	0.373	0.259	0.059	0.06	0.212	0.158
	Anderson-Rubin test (p-value)	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000
	N	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128
Cross-Section	Visited CM	0.671*** (0.028) [0.001]	0.672*** (0.027) [0.001]	2.027*** (0.226) [0.001]	2.188*** (0.201) [0.001]	0.362*** (0.027) [0.001]	0.349*** (0.027) [0.001]	0.642*** (0.152) [0.001]	0.721*** (0.133) [0.001]
	Control Group's Mean	0.084	0.074	0.373	0.259	0.059	0.060	0.212	0.158
	Anderson-Rubin test (p-value)	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000
	N	2,579	2,564	2,579	2,564	2,579	2,564	2,579	2,564

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

Cross-Section: Baseline covariates included; age, head of household, has a couple, couple lives in the household, N rooms in household, N children, N children < 5 living in household, literate, years of education, socioeconomic index, owner of house, employed, labor income, affiliated to a public or private social security system.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table 6. Impact on Use of Social Services and Life Satisfaction

		Instrumental Variables Second Stage													
		(I) Aggregate index of public services (z- score)		(I.1) Index of sexual and reproductive health services (z-score)		(I.2) Index of services to promote female employment (z- score)		(I.3) Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence (z-score)		(I.4) Index of legal services for victims of patrimonial violence (z-score)		(I.5) Index of legal services to strengthen economic autonomy (z-score)		(II) Very Satisfied/Satisfied with life in general	
		CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Fixed Effects	Visited CM	0.525*** (0.119) [0.001]	0.516*** (0.113) [0.001]	0.288*** (0.110) [0.047]	0.484*** (0.101) [0.001]	0.257** (0.110) [0.047]	0.126 (0.131) [0.509]	-0.061 (0.125) [0.650]	-0.105 (0.110) [0.509]	0.481*** (0.128) [0.001]	0.290** (0.124) [0.088]	0.399*** (0.154) [0.047]	0.346*** (0.131) [0.044]	0.107** (0.043) [0.047]	0.083* (0.044) [0.175]
	Control Group's Mean	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.843	0.851
	Anderson-Rubin p-val	0.000	0.000	0.008	0.000	0.020	0.341	0.625	0.340	0.000	0.018	0.008	0.008	0.012	0.054
	N	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128
Cross- Section	Visited CM	0.376*** (0.096) [0.012]	0.287*** (0.094) [0.091]	0.296*** (0.090) [0.017]	0.370*** (0.086) [0.001]	0.147 (0.093) [0.374]	0.077 (0.095) [0.774]	-0.023 (0.096) [0.990]	-0.122 (0.093) [0.774]	0.376*** (0.117) [0.017]	0.189* (0.103) [0.377]	-0.006 (0.125) [0.990]	-0.086 (0.092) [0.774]	0.061* (0.036) [0.580]	0.043 (0.032) [0.774]
	Control Group's Mean	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.843	0.851
	Anderson-Rubin p-val	0.000	0.002	0.001	0.000	0.117	0.422	0.807	0.190	0.001	0.065	0.963	0.348	0.087	0.188
	N	2,579	2,564	2,579	2,564	2,579	2,564	2,579	2,564	2,579	2,564	2,579	2,564	2,579	2,564

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Index of sexual and reproductive health services: Prenatal checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Dental checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Postnatal checkup in the last 12 months, women who have given birth in last 12 months; Citology/Papanicolaou in the last 12 months; Mammography in the last 12 months, women older than 40 years old.

Index of services to promote female employment: Job training in the last 12 months; Registration for job opportunities, assistance to prepare CV or help for job interview in the last 12 months,

Independent or unemployed women; Orientation to start or expand the business in the last 12 months, Independent or unemployed women; Credit/monetary support to open or expand business in the last 12 months, Independent or unemployed women who applied to a credit or requested monetary support.

Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence: Sought/received emotional support; Sought/received legal support; Sought/received injunction for protection; Sought/received medical aid; Sought/received transportation support; Sought/received support to put a complaint.

Index of legal services for victims of patrimonial violence: Has received help for acknowledgment of paternity of their children, Women with children; Has received help for legalization of property/assets; Has received help for food fees, Women with children.

Index of legal services to strengthen economic autonomy: Has received help to obtain her ID; Has received help to obtain her birth certificate.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

Cross-Section baseline covariates included; age, head of household, has a couple, couple lives in the household, N rooms in household, N children, N children < 5 living in household, literate, years of education, socioeconomic index, owner of house, employed, labor income, affiliated to a public or private social security system, and the dependent variable in baseline.

All observations weighted by the inverse probability of being selected in the sample (IPW). Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values by Romano and Wolf (2015) in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table 7. Compliers and Non-Compliers Baseline Sociodemographic Profile

	CM vs. HU					CM vs. NP				
	Mean			Difference		Mean			Difference	
	Compliers (N = 1,228)	Always-Takers (N = 406)	Never-Takers (N = 945)	C - AT	C - NT	Compliers (N = 1,283)	Always-Takers (N = 342)	Never-Takers (N = 939)	C - AT	C - NT
Age	38.246 (0.477)	36.554 (0.801)	36.179 (0.545)	1.692*	2.067***	38.702 (0.456)	35.160 (0.883)	36.179 (0.522)	3.542***	2.523***
Head household	0.288 (0.019)	0.282 (0.031)	0.238 (0.019)	0.006 (0.036)	0.050* (0.027)	0.273 (0.018)	0.284 (0.034)	0.238 (0.019)	-0.011 (0.038)	0.035 (0.026)
Married or with a partner	0.625 (0.020)	0.619 (0.034)	0.642 (0.022)	0.006 (0.039)	-0.017 (0.030)	0.651 (0.019)	0.598 (0.037)	0.642 (0.022)	0.053 (0.042)	0.009 (0.029)
Partner lives in the hh	0.603 (0.020)	0.619 (0.036)	0.619 (0.023)	-0.016 (0.041)	-0.016 (0.030)	0.636 (0.019)	0.574 (0.039)	0.619 (0.023)	0.062 (0.043)	0.017 (0.030)
N alive born children	2.520 (0.079)	2.604 (0.138)	2.225 (0.093)	-0.084 (0.159)	0.295** (0.122)	2.683 (0.076)	2.320 (0.147)	2.225 (0.098)	0.363** (0.165)	0.458*** (0.124)
N children 0-5 in hh	0.340 (0.025)	0.366 (0.042)	0.352 (0.028)	-0.026 (0.049)	-0.012 (0.038)	0.334 (0.022)	0.343 (0.040)	0.352 (0.026)	-0.009 (0.046)	-0.018 (0.034)
N children 6-12 in hh	0.510 (0.028)	0.574 (0.053)	0.451 (0.032)	-0.064 (0.060)	0.059 (0.043)	0.533 (0.028)	0.438 (0.055)	0.451 (0.033)	0.095 (0.062)	0.082* (0.043)
Literate	0.889 (0.012)	0.941 (0.016)	0.920 (0.012)	-0.052*** (0.020)	-0.031* (0.017)	0.884 (0.012)	0.893 (0.024)	0.920 (0.013)	-0.009 (0.027)	-0.036** (0.018)
Years of education	7.314 (0.197)	7.272 (0.287)	8.815 (0.228)	0.042 (0.348)	-1.501*** (0.301)	7.009 (0.189)	7.503 (0.356)	8.815 (0.238)	-0.494 (0.403)	-1.806*** (0.304)
Employed	0.619 (0.020)	0.599 (0.034)	0.568 (0.022)	0.020 (0.039)	0.051* (0.030)	0.578 (0.020)	0.538 (0.038)	0.568 (0.022)	0.040 (0.043)	0.010 (0.030)
Labor income (IHS)	3.407 (0.128)	3.239 (0.208)	3.344 (0.145)	0.168 (0.244)	0.063 (0.193)	3.201 (0.117)	2.886 (0.232)	3.344 (0.146)	0.315 (0.260)	-0.143 (0.187)
Affiliated to soc. sec. system	0.127 (0.015)	0.059 (0.017)	0.221 (0.019)	0.068*** (0.023)	-0.094*** (0.024)	0.100 (0.013)	0.065 (0.019)	0.221 (0.018)	0.035 (0.023)	-0.121*** (0.022)
Socioeconomic index	0.191 (0.009)	0.198 (0.016)	0.153 (0.010)	-0.007 (0.018)	0.038*** (0.013)	0.206 (0.009)	0.215 (0.017)	0.153 (0.010)	-0.009 (0.019)	0.053*** (0.013)
Household owner	0.596 (0.020)	0.609 (0.033)	0.600 (0.022)	-0.013 (0.039)	-0.004 (0.030)	0.611 (0.019)	0.556 (0.038)	0.600 (0.023)	0.055 (0.042)	0.011 (0.030)

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion. Socioeconomic index: Has a substandard housing (it is not a private or independent house, apartment, or condominium); Materials of the walls are precarious (it is not concrete, brick, or wood); Housing has no water; Housing has no electricity; No private toilet-flush water. Decomposition following Marbach and Hangartner (2020). Bootstrap standard errors in parentheses (reps. = 1,000). *** p<0.01, ** p<0.05, * p<0.1

Table 8. Service Use in Baseline for Compliers and Non-Compliers

	CM vs. HU					CM vs. NP				
	Mean			Difference		Mean			Difference	
	Compliers (N = 1,228)	Always-Takers (N = 406)	Never-Takers (N = 945)	C - AT	C - NT	Compliers (N = 1,283)	Always-Takers (N = 342)	Never-Takers (N = 939)	C - AT	C - NT
Aggregate index of public services	0.021 (0.039)	0.188 (0.086)	-0.098 (0.042)	-0.167* (0.094)	0.119** (0.057)	-0.060 (0.036)	0.126 (0.082)	-0.156 (0.040)	-0.186** (0.090)	0.096* (0.054)
Index of sexual and reproductive health services	0.121 (0.042)	0.038 (0.072)	0.011 (0.043)	0.083 (0.083)	0.110* (0.060)	0.002 (0.038)	0.119 (0.077)	-0.074 (0.040)	-0.117 (0.086)	0.076 (0.055)
Index of services to promote female employment	-0.042 (0.044)	0.148 (0.099)	0.026 (0.053)	-0.190* (0.108)	-0.068 (0.069)	-0.022 (0.038)	0.017 (0.082)	0.013 (0.045)	-0.039 (0.090)	-0.035 (0.059)
Index of psychological, medical and legal services for survivors of physical, sexual, and/or emotional violence	-0.023 (0.038)	0.275 (0.108)	-0.142 (0.029)	-0.298** (0.114)	0.119** (0.048)	-0.006 (0.036)	0.169 (0.090)	-0.105 (0.032)	-0.175* (0.097)	0.099** (0.048)
Index of legal services for victims of patrimonial violence	-0.027 (0.039)	0.054 (0.085)	-0.041 (0.042)	-0.081 (0.094)	0.014 (0.057)	-0.060 (0.036)	0.040 (0.069)	-0.030 (0.043)	-0.100 (0.078)	-0.030 (0.056)
Index of legal services to strengthen economic autonomy	-0.066 (0.035)	-0.002 (0.068)	-0.170 (0.032)	-0.064 (0.076)	0.104** (0.047)	-0.094 (0.032)	-0.041 (0.072)	-0.210 (0.030)	-0.053 (0.079)	0.116** (0.044)
Very Satisfied/Satisfied with life in general	0.834 (0.017)	0.748 (0.030)	0.802 (0.017)	0.086** (0.034)	0.032 (0.024)	0.823 (0.016)	0.728 (0.034)	0.802 (0.019)	0.095** (0.038)	0.021 (0.025)

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Index of sexual and reproductive health services: Prenatal checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Dental checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Postnatal checkup in the last 12 months, women who have given birth in last 12 months; Citology/Papanicolaou in the last 12 months; Mammography in the last 12 months, Women older than 40 years old.

Index of services to promote female employment: Job training in the last 12 months; Registration for job opportunities, assistance to prepare CV or help for job interview in the last 12 months, Independent or unemployed women; Orientation to start or expand the business in the last 12 months, Independent or unemployed women; Credit/monetary support to open or expand business in the last 12 months, Independent or unemployed women who applied to a credit or requested monetary support.

Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence: Sought/received emotional support; Sought/received legal support; Sought/received injunction for protection; Sought/received medical aid; Sought/received transportation support; Sought/received support to put a complaint.

Index of legal services for victims of patrimonial violence: Has received help for acknowledgment of paternity of their children, Women with children; Has received help for legalization of property/assets; Has received help for food fees, Women with children.

Index of legal services to strengthen economic autonomy: Has received help to obtain her ID; Has received help to obtain her birth certificate.

Decomposition following Marbach and Hangartner (2020). Bootstrap standard errors in parentheses (reps. = 1,000). *** p<0.01, ** p<0.05, * p<0.1

Table 9. Impacts on Demand. Heterogeneous Effects

Instrumental Variables Fixed Effects Second Stage								
	(I)		(II)		(III)		(IV)	
	Probability to have a subsequent visit to CM		Number of subsequent visits to CM		Probability to have a non-reproductive health subsequent visit to CM		Number of non-reproductive health subsequent visits to CM	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM × Socioeconomic Index	0.095 (0.137)	0.170 (0.115)	0.701 (1.051)	1.081 (0.870)	0.119 (0.135)	0.165 (0.124)	0.155 (0.722)	0.422 (0.619)
Visited CM × Years of Educ.	-0.009 (0.006)	-0.013** (0.006)	-0.064* (0.037)	-0.099*** (0.038)	-0.010* (0.006)	-0.014** (0.006)	-0.019 (0.022)	-0.046 (0.030)
Visited CM × Years of Age	-0.004 (0.002)	-0.001 (0.002)	0.018 (0.014)	0.023* (0.013)	0.001 (0.002)	0.003 (0.002)	0.001 (0.009)	0.005 (0.008)
Visited CM × N Children (0-12) in HH	0.035 (0.028)	0.040 (0.025)	-0.319 (0.227)	-0.201 (0.175)	0.014 (0.029)	0.034 (0.026)	-0.225 (0.155)	-0.045 (0.118)
Control Group's Mean	0.084	0.074	0.373	0.259	0.059	0.060	0.212	0.158
N	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Socioeconomic index: Has a substandard housing (it is not a private or independent house, apartment, or condominium); Materials of the walls are precarious (it is not concrete, brick, or wood); Housing has no water; Housing has no electricity; No private toilet-flush water.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. *** p<0.01, ** p<0.05, * p<0.1

Table 10. Impact on Use of Social Services and Life Satisfaction. Heterogeneous Effects

Instrumental Variables Fixed Effects Second Stage														
	(I)		(I.1)		(I.2)		(I.3)		(I.4)		(I.5)		(II)	
	Aggregate index of public services (z-score)		Index of sexual and reproductive health services (z-score)		Index of services to promote female employment (z-score)		Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence (z-score)		Index of legal services for victims of patrimonial violence (z-score)		Index of legal services to strengthen economic autonomy (z-score)		Very Satisfied/Satisfied with life in general	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM × Socioeconomic Index	-0.707 (0.499)	0.026 (0.483)	-0.361 (0.506)	0.072 (0.461)	-0.503 (0.500)	0.696 (0.640)	-0.187 (0.553)	0.107 (0.400)	-0.138 (0.523)	-0.294 (0.565)	-0.476 (0.525)	-0.375 (0.496)	-0.044 (0.223)	-0.072 (0.202)
Visited CM × Years of Educ.	0.008 (0.025)	0.017 (0.028)	-0.019 (0.023)	-0.013 (0.023)	0.017 (0.024)	0.029 (0.035)	-0.029 (0.029)	0.014 (0.029)	0.030 (0.027)	-0.003 (0.027)	0.003 (0.040)	0.029 (0.028)	-0.005 (0.010)	0.000 (0.011)
Visited CM × Years of Age	0.023*** (0.009)	0.017** (0.008)	0.027*** (0.009)	0.021** (0.008)	0.007 (0.009)	0.007 (0.012)	0.012* (0.007)	0.011 (0.007)	-0.002 (0.009)	-0.005 (0.009)	-0.002 (0.015)	-0.006 (0.012)	0.006* (0.004)	0.002 (0.004)
Visited CM × N Children (0-12) in HH	-0.109 (0.112)	-0.073 (0.110)	-0.074 (0.121)	-0.037 (0.105)	-0.066 (0.103)	0.036 (0.131)	-0.067 (0.119)	-0.108 (0.107)	0.082 (0.133)	0.002 (0.118)	-0.007 (0.146)	-0.059 (0.120)	-0.094** (0.043)	-0.015 (0.043)
Control Group's Mean	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.000	0.843	0.851
N	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Index of sexual and reproductive health services: Prenatal checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Dental checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Postnatal checkup in the last 12 months, women who have given birth in last 12 months; Citology/Papanicolaou in the last 12 months; Mammography in the last 12 months, Women older than 40 years old.

Index of services to promote female employment: Job training in the last 12 months; Registration for job opportunities, assistance to prepare CV or help for job interview in the last 12 months, Independent or unemployed women; Orientation to start or expand the business in the last 12 months, Independent or unemployed women; Credit/monetary support to open or expand business in the last 12 months, Independent or unemployed women who applied to a credit or requested monetary support.

Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence: Sought/received emotional support; Sought/received legal support; Sought/received injunction for protection; Sought/received medical aid; Sought/received transportation support; Sought/received support to put a complaint.

Index of legal services for victims of patrimonial violence: Has received help for acknowledgment of paternity of their children, Women with children; Has received help for legalization of property/assets; Has received help for food fees, Women with children.

Index of legal services to strengthen economic autonomy: Has received help to obtain her ID; Has received help to obtain her birth certificate.

Socioeconomic index: Has a substandard housing (it is not a private or independent house, apartment, or condominium); Materials of the walls are precarious (it is not concrete, brick, or wood); Housing has no water; Housing has no electricity; No private toilet-flush water.

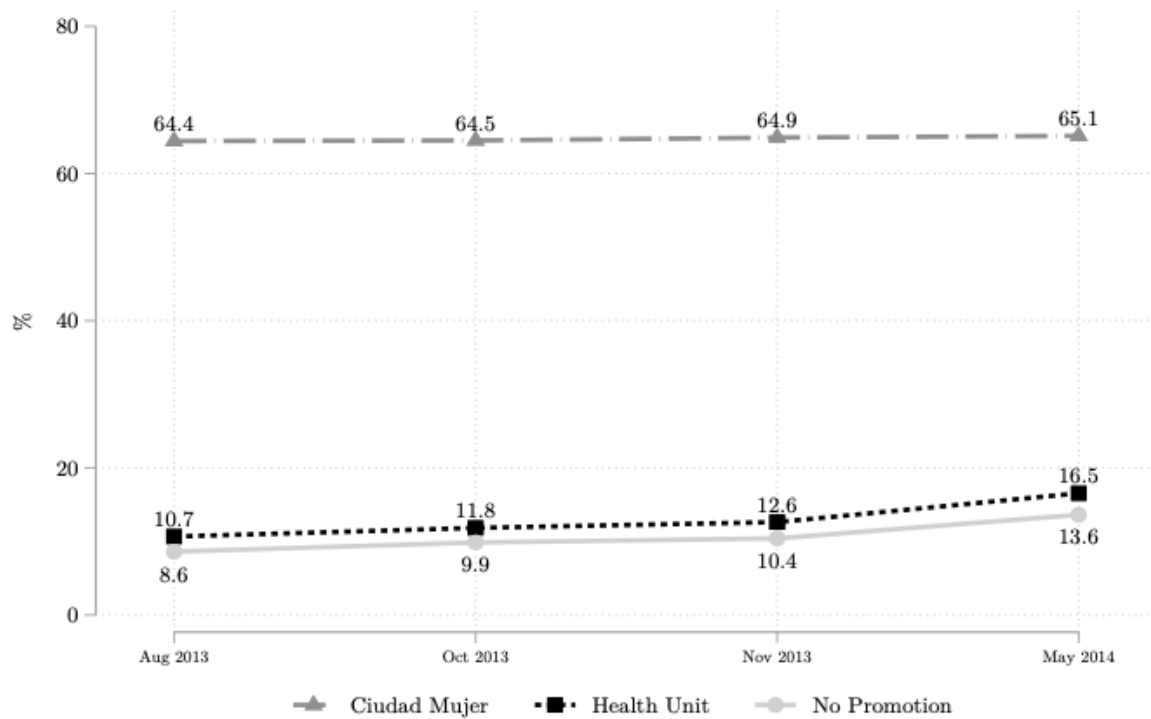
In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. *** p<0.01, ** p<0.05, * p<0.1

FIGURES

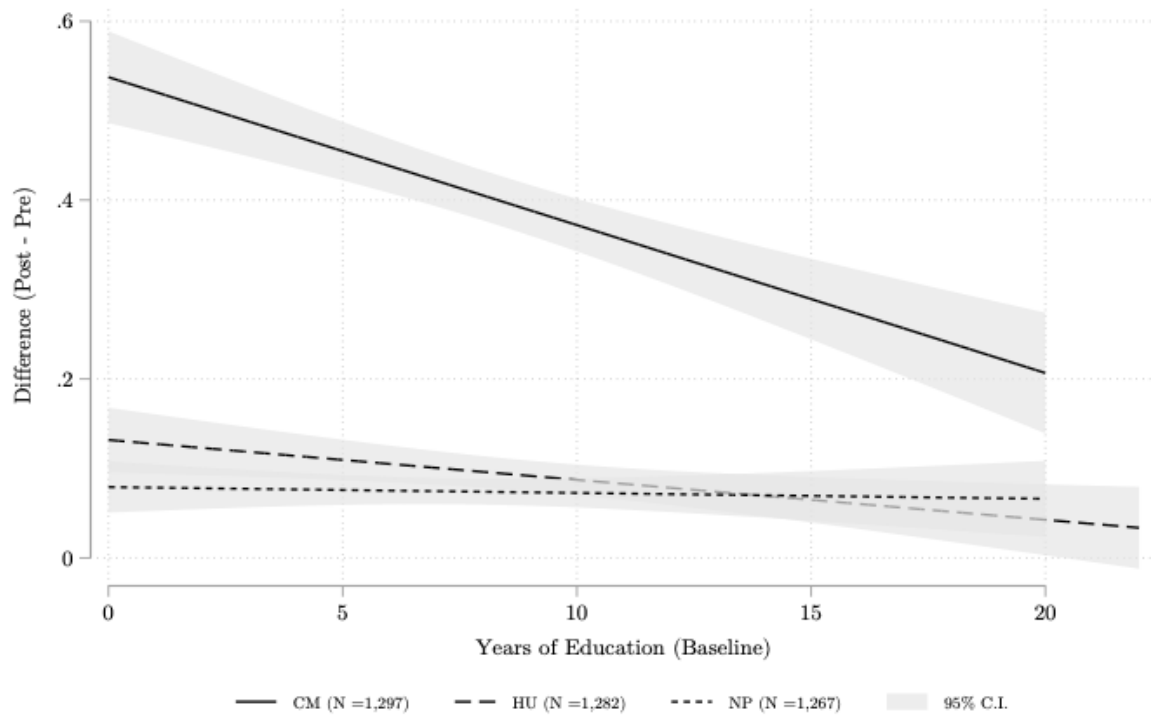
Figure 1. Take-up Rate. Proportion of Women Who Made an Initial Visit to CM Centers



SOURCE: Ciudad Mujer Centers' administrative data.

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion. Source: Administrative data from Ciudad Mujer centers.

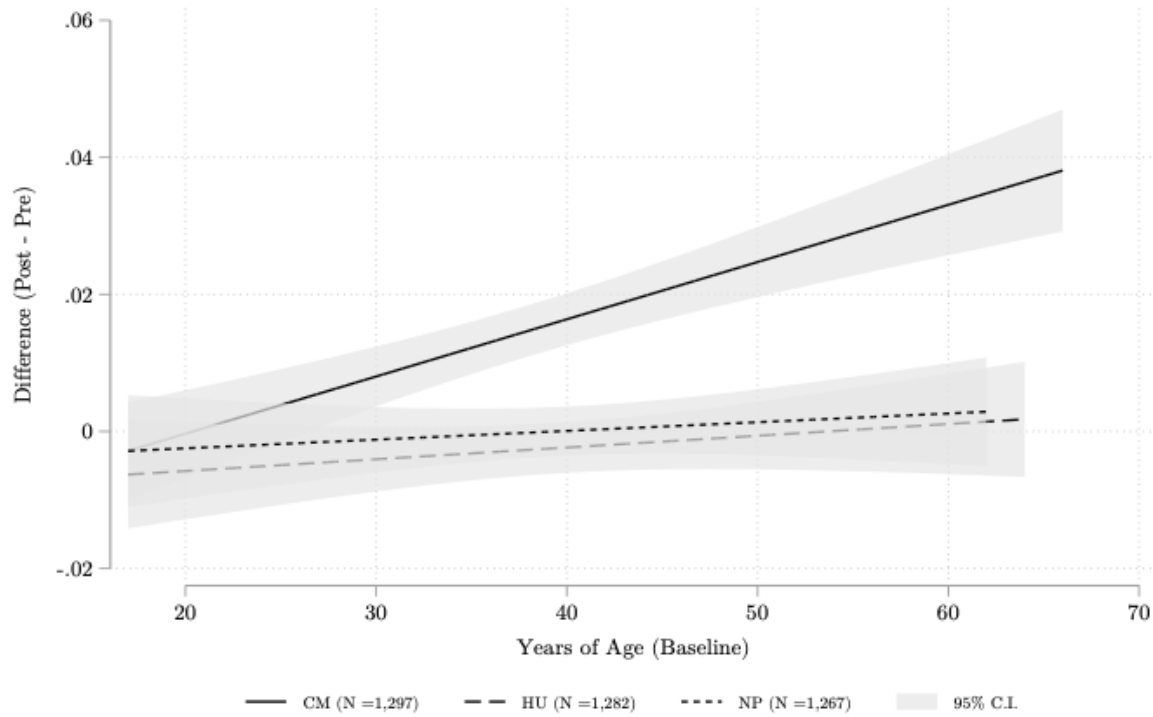
Figure 2. ITT on The Probability of Having a Subsequent Visit to CM and Women's Education at Baseline



Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Each fitted line represents the change in the outcome by assignment group. The y-axis shows the second- and first-period outcome difference, so the vertical distance between the solid line and the two other dashed lines represents the corresponding FE (ITT) estimator across the values of the heterogeneity variable.

Figure 3. ITT on Aggregate Services' Index and Women's Age at Baseline



Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Each fitted line represents the change in the outcome by assignment group. The y-axis shows the second- and first-period outcome difference, so the vertical distance between the solid line and the two other dashed lines represents the corresponding FE (ITT) estimator across the values of the heterogeneity variable.

APPENDIX

Table A1. Sample Composition and Attrition

	Promotion Group			Total
	CM	HU	NP	
Original selected sample	1,538	1,539	1,541	4,618
Base Line	1,354	1,353	1,355	4,062
Follow-Up	1,297	1,282	1,267	3,846
Attrition (%)	4.2	5.2	6.5	5.3
		[0.877]	[0.208]	

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Values in brackets are p-values from comparisons of attrition rates with the CM arm, estimated using an IPW-weighted OLS specification with standard errors clustered at the PSU level.

Table A2. Intention-to-Treat (ITT) Impacts on Demand

	Fixed Effects Model			
	(I) Probability to have a subsequent visit to CM	(II) Number of subsequent visits to CM	(III) Probability to have a non- reproductive health subsequent visit to CM	(IV) Number of non- reproductive health subsequent visits to CM
Promoted CM	0.317*** (0.026) [0.001]	1.027*** (0.143) [0.001]	0.167*** (0.021) [0.001]	0.334*** (0.088) [0.001]
Promoted HU	0.010 (0.015) [0.615]	0.113 (0.098) [0.368]	-0.001 (0.013) [0.923]	0.054 (0.068) [0.615]
Control Group's Mean (NP)	0.074	0.259	0.060	0.158
N	7,692	7,692	7,692	7,692

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values by Romano and Wolf (2015) in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A3. Intention-to-Treat (ITT) Impacts on Use of Services and Life Satisfaction

Fixed Effects Model							
(I)	(I.1)	(I.2)	(I.3)	(I.4)	(I.5)	(II)	
Aggregate index of public services (z-score)	Index of sexual and reproductive health services (z-score)	Index of services to promote female employment (z-score)	Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence (z-score)	Index of legal services for victims of patrimonial violence (z-score)	Index of legal services to strengthen economic autonomy (z-score)	Very Satisfied/Satisfied with life in general	
Promoted CM	0.242*** (0.073) [0.002]	0.227*** (0.065) [0.001]	0.059 (0.088) [0.621]	-0.051 (0.074) [0.621]	0.136* (0.082) [0.169]	0.162* (0.086) [0.114]	0.039 (0.029) [0.264]
Promoted HU	0.020 (0.070) [0.979]	0.100 (0.067) [0.355]	-0.062 (0.085) [0.885]	-0.027 (0.062) [0.914]	-0.061 (0.074) [0.873]	-0.006 (0.103) [0.979]	-0.009 (0.031) [0.979]
Control Group's Mean (NP)	0.000	0.000	0.000	0.000	0.000	0.000	0.851
N	7,692	7,692	7,692	7,692	7,692	7,692	7,692

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Index of sexual and reproductive health services: Prenatal checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Dental checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Postnatal checkup in the last 12 months, women who have given birth in last 12 months; Citology/Papanicolaou in the last 12 months; Mammography in the last 12 months, women older than 40 years old.

Index of services to promote female employment: Job training in the last 12 months; Registration for job opportunities, assistance to prepare CV or help for job interview in the last 12 months, Independent or unemployed women; Orientation to start or expand the business in the last 12 months, Independent or unemployed women; Credit/monetary support to open or expand business in the last 12 months, Independent or unemployed women who applied to a credit or requested monetary support.

Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence: Sought/received emotional support; Sought/received legal support; Sought/received injunction for protection; Sought/received medical aid; Sought/received transportation support; Sought/received support to put a complaint.

Index of legal services for victims of patrimonial violence: Has received help for acknowledgment of paternity of their children, Women with children; Has received help for legalization of property/assets; Has received help for food fees, Women with children.

Index of legal services to strengthen economic autonomy: Has received help to obtain her ID; Has received help to obtain her birth certificate.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A4. Impact on Use of Sexual and Reproductive Health Services

Instrumental Variables FE Model Second Stage										
	(I) Cytology/Pap in the last 12 months		(II) Mammography in the last 12 months		(III) Prenatal checkup in the last 12 months		(IV) Dental checkup in the last 12 months		(V) Postnatal checkup in the last 12 months	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM	0.190*** (0.054) [0.007]	0.257*** (0.051) [0.001]	0.144*** (0.028) [0.001]	0.143*** (0.028) [0.001]	-0.069** (0.028) [0.037]	-0.022 (0.027) [0.434]	-0.056** (0.024) [0.037]	-0.029 (0.020) [0.244]	0.013 (0.022) [0.555]	0.050* (0.026) [0.127]
Control Group's Mean	0.563	0.559	0.069	0.067	0.057	0.051	0.041	0.036	0.026	0.030
Anderson-Rubin test (p-value)	0.000	0.000	0.000	0.000	0.013	0.429	0.019	0.144	0.565	0.048
N	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A5. Impact on Use of Legal Services for Victims of Patrimonial Violence

	Instrumental Variables FE Model Second Stage					
	(I)		(II)		(III)	
	Received help for paternity acknowledgment of children in the last 12 months		Received help for food fees in the last 12 months		Received help for legalization of property/assets in the last 12 months	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM	0.053*** (0.019) [0.018]	0.032* (0.017) [0.182]	0.028 (0.018) [0.117]	0.033 (0.020) [0.210]	0.048*** (0.017) [0.018]	0.022 (0.020) [0.278]
Control Group's Mean	0.012	0.014	0.022	0.026	0.013	0.020
Anderson-Rubin test (p-value)	0.005	0.063	0.120	0.103	0.004	0.267
N	5,158	5,128	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A6. Impact on Use of Services to Strength Economic Autonomy

Instrumental Variables FE Model Second Stage				
	(I)		(II)	
	Received help to obtain her ID in the last 12 months		Received help to obtain her birth certificate in the last 12 months	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM	0.124*** (0.032) [0.001]	0.113*** (0.034) [0.001]	0.058** (0.029) [0.044]	0.076*** (0.029) [0.009]
Control Group's Mean	0.026	0.040	0.026	0.030
Anderson-Rubin test (p-value)	0.000	0.001	0.043	0.008
N	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A7. Impact on Use of Psychological, Medical, and Legal Services for Survivors of Physical, Sexual, and/or Emotional Violence

Instrumental Variables FE Model Second Stage												
	(I) Sought/received emotional support		(II) Sought/received legal support		(III) Sought/received injunction for protection		(IV) Sought/received medical aid		(V) Sought/received transportation support		(VI) Sought/received support to put a complaint	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM	0.030 (0.020) [0.503]	0.006 (0.020) [0.958]	-0.020 (0.021) [0.735]	-0.023 (0.020) [0.698]	-0.006 (0.011) [0.814]	-0.008 (0.015) [0.926]	-0.012 (0.012) [0.735]	-0.017 (0.014) [0.682]	-0.011 (0.009) [0.643]	-0.002 (0.009) [0.958]	-0.011 (0.020) [0.814]	-0.041* (0.021) [0.249]
Control Group's Mean	0.018	0.023	0.022	0.027	0.011	0.020	0.007	0.012	0.008	0.004	0.024	0.037
Anderson-Rubin test (p-value)	0.128	0.776	0.354	0.269	0.579	0.580	0.313	0.224	0.207	0.803	0.582	0.052
N	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A8. Impact on Use of Services to Promote Female Employment

Instrumental Variables FE Model Second Stage								
	(I)		(II)		(III)		(IV)	
	Requested/received job training in the last 12 months		Requested/received job placement services in the last 12 months		Requested/received orientation to start or improve own business in the last 12 months		Requested/received credit/monetary support to open or expand business in the last 12 months	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM	0.056*** (0.018) [0.007]	0.037 (0.024) [0.411]	-0.005 (0.020) [0.798]	0.002 (0.024) [0.967]	0.033*** (0.012) [0.021]	0.018 (0.014) [0.474]	0.016 (0.026) [0.781]	-0.007 (0.025) [0.967]
Control Group's Mean	0.024	0.037	0.037	0.031	0.010	0.016	0.078	0.071
Anderson-Rubin test (p-value)	0.002	0.127	0.805	0.936	0.006	0.194	0.535	0.796
N	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A9. Intention-to-Treat (ITT) Impacts on Demand. Heterogeneous Effects

	Fixed Effects Model			
	(I) Probability to have a subsequent visit to CM	(II) Number of subsequent visits to CM	(III) Probability to have a non- reproductive health subsequent visit to CM	(IV) Number of non- reproductive health subsequent visits to CM
Promoted CM × Socioeconomic Index	0.216* (0.112)	0.956 (0.651)	0.150 (0.094)	0.345 (0.436)
Promoted HU × Socioeconomic Index	0.077 (0.080)	0.359 (0.456)	0.045 (0.065)	0.189 (0.312)
Promoted CM × Years of Educ.	-0.017*** (0.005)	-0.077*** (0.025)	-0.012*** (0.004)	-0.031* (0.018)
Promoted HU × Years of Educ.	-0.004 (0.004)	-0.024 (0.026)	-0.002 (0.003)	-0.015 (0.017)
Promoted CM × Years of Age	0.003 (0.002)	0.022** (0.010)	0.003* (0.002)	0.006 (0.006)
Promoted HU × Years of Age	0.002 (0.001)	0.005 (0.008)	0.002 (0.001)	0.003 (0.006)
Promoted CM × N Children	0.040 (0.026)	-0.039 (0.136)	0.028 (0.023)	-0.002 (0.086)
Promoted HU × N Children	0.031* (0.018)	0.134 (0.126)	0.025 (0.016)	0.111 (0.089)
Control Group's Mean (NP)	0.074	0.259	0.060	0.158
N	7,692	7,692	7,692	7,692

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Socioeconomic index: Has a substandard housing (it is not a private or independent house, apartment, or condominium); Materials of the walls are precarious (it is not concrete, brick, or wood); Housing has no water; Housing has no electricity; No private toilet-flush water.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. *** p<0.01, ** p<0.05, * p<0.1

Table A10. Intention-to-Treat (ITT) Impacts on Use of Services and Life Satisfaction. Heterogeneous Effects

	(I)	(I.1)	(I.2)	(I.3)	(I.4)	(I.5)	(II)
	Aggregate index of public services (z-score)	Index of sexual and reproductive health services (z-score)	Index of services to promote female employment (z-score)	Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence (z-score)	Index of legal services for victims of patrimonial violence (z-score)	Index of legal services to strengthen economic autonomy (z-score)	Very Satisfied/Satisfied with life in general
Promoted CM × Socioeconomic Index	0.114 (0.321)	0.130 (0.307)	0.361 (0.448)	0.029 (0.259)	-0.085 (0.381)	-0.114 (0.331)	-0.018 (0.139)
Promoted HU × Socioeconomic Index	0.339 (0.295)	0.251 (0.302)	0.563 (0.471)	0.104 (0.229)	-0.127 (0.351)	0.037 (0.358)	-0.014 (0.161)
Promoted CM × Years of Educ.	-0.002 (0.016)	-0.014 (0.013)	0.010 (0.019)	0.008 (0.017)	-0.006 (0.015)	0.006 (0.015)	-0.001 (0.007)
Promoted HU × Years of Educ.	0.008 (0.016)	-0.002 (0.012)	0.005 (0.018)	0.018 (0.016)	-0.009 (0.014)	0.009 (0.022)	0.002 (0.007)
Promoted CM × Years of Age	0.011* (0.005)	0.012** (0.005)	0.004 (0.008)	0.004 (0.004)	-0.001 (0.006)	-0.001 (0.008)	0.001 (0.002)
Promoted HU × Years of Age	-0.001 (0.006)	-0.001 (0.005)	-0.001 (0.007)	-0.001 (0.004)	-0.002 (0.006)	-0.001 (0.010)	-0.002 (0.003)
Promoted CM × N Children	-0.022 (0.081)	-0.005 (0.076)	0.022 (0.095)	-0.055 (0.078)	0.009 (0.085)	-0.020 (0.083)	-0.005 (0.031)
Promoted HU × N Children	0.032 (0.078)	0.032 (0.072)	0.053 (0.094)	-0.033 (0.081)	-0.013 (0.076)	-0.012 (0.096)	0.041 (0.034)
Control Group's Mean (NP)	0.000	0.000	0.000	0.000	0.000	0.000	0.851
N	7,692	7,692	7,692	7,692	7,692	7,692	7,692

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Index of sexual and reproductive health services: Prenatal checkup in the last 12 months, women who have given birth in last 12 months or currently pregnant; Dental checkup in the last 12 months, women who have given birth in last 12 months; Citology/Papanicolaou in the last 12 months; Mammography in the last 12 months, Women older than 40 years old.

Index of services to promote female employment: Job training in the last 12 months; Registration for job opportunities, assistance to prepare CV or help for job interview in the last 12 months, Independent or unemployed women; Orientation to start or expand the business in the last 12 months, Independent or unemployed women; Credit/monetary support to open or expand business in the last 12 months, Independent or unemployed women who applied to a credit or requested monetary support.

Index of psychological, medical, and legal services for survivors of physical, sexual, and/or emotional violence: Sought/received emotional support; Sought/received legal support; Sought/received injunction for protection; Sought/received medical aid;

Sought/received transportation support; Sought/received support to put a complaint.

Index of legal services for victims of patrimonial violence: Has received help for acknowledgment of paternity of their children, Women with children; Has received help for legalization of property/assets; Has received help for food fees, Women with children.

Index of legal services to strengthen economic autonomy: Has received help to obtain her ID; Has received help to obtain her birth certificate.

Socioeconomic index: Has a substandard housing (it is not a private or independent house, apartment, or condominium); Materials of the walls are precarious (it is not concrete, brick, or wood); Housing has no water; Housing has no electricity; No private toilet-flush water.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. *** p<0.01, ** p<0.05, * p<0.1

Table A11. Impact on Reproductive Health Outcomes

	Instrumental Variables FE Model Second Stage			
	(I)		(II)	
	Uses any contraceptive method (women between 15-49 years of age, sexually active, and not looking for pregnancy)		Uses a modern contraceptive method (women between 15-49 years of age, sexually active, and not looking for pregnancy)	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM	0.054 (0.050) [0.148]	0.007 (0.042) [0.512]	0.011 (0.041) [0.148]	-0.010 (0.040) [0.512]
Control Group's Mean	0.802	0.836	0.265	0.304
Anderson-Rubin test (p-value)	0.279	0.869	0.786	0.797
N	2,912	2,862	2,912	2,862

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A12. Impact on Health Outcomes

	Instrumental Variables (Cross-Section) Second Stage									
	(I)		(II)		(III)		(IV)		(V)	
	Follows any treatment: mammogram, hypertension, diabetes, overweight/obesity		Received treatment after the problem detected in the last mammogram		Follows treatment for hypertension		Follows treatment for diabetes		Follows treatment to control overweight/obesity	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Visited CM	0.020	-0.006	0.000	-0.002	0.000	-0.008	-0.005	-0.015*	-0.029	0.004
	(0.029)	(0.028)	(0.008)	(0.007)	(0.021)	(0.021)	(0.010)	(0.009)	(0.023)	(0.021)
	[0.962]	[0.993]	[1.000]	[0.993]	[1.000]	[0.993]	[0.962]	[0.409]	[0.637]	[0.993]
Control Group's Mean	0.209	0.222	0.017	0.019	0.130	0.145	0.049	0.052	0.094	0.082
Anderson-Rubin test (p-value)	0.505	0.825	0.976	0.818	0.992	0.712	0.602	0.087	0.208	0.841
N	2,579	2,564	2,579	2,564	2,579	2,564	2,579	2,564	2,579	2,564

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

We report cross-section estimates (instead of FE model) in this table because the information of these five outcomes was only collected in the follow-up survey.

Baseline covariates included; age, head of household, has a couple, couple lives in the household, N rooms in household, N children, N children < 5 living in household, literate, years of education, socioeconomic index, owner of house, employed, labor income, affiliated to a public or private social security system, and an indicator for being diagnosed with the problem/disease in each column.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A13. Impact on Labor Outcomes

Instrumental Variables FE Model Second Stage						
	(I) Employment		(II) IHS Labor Income (Unconditional)		(III) Formal Work (Unconditional)	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Attended CM	0.079*	0.014	0.441*	0.169	0.041*	0.041*
	(0.045)	(0.042)	(0.263)	(0.240)	(0.022)	(0.023)
	[0.158]	[0.732]	[0.158]	[0.618]	[0.158]	[0.173]
Control Group's Mean	0.601	0.575	3.322	3.199	0.170	0.159
Anderson-Rubin test (p-value)	0.077	0.734	0.092	0.482	0.064	0.078
N	5,158	5,128	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A14. Impact on Intimate Partner Violence

Instrumental Variables FE Model Second Stage										
	(I) Violence Phys/Sex/Emo/Pat, Couple/Ex, Last 12 months		(II) Violence Phys, Couple/Ex, Last 12 months		(III) Violence Sex, Couple/Ex, Last 12 months		(IV) Violence Emo, Couple/Ex, Last 12 months		(V) Violence Pat, Couple/Ex, Last 12 months	
	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP	CM vs. HU	CM vs. NP
Attended CM	-0.031 (0.025) [0.514]	0.026 (0.027) [0.796]	-0.001 (0.014) [0.944]	-0.004 (0.017) [0.968]	-0.019* (0.010) [0.235]	0.003 (0.010) [0.968]	-0.029 (0.023) [0.514]	0.014 (0.024) [0.938]	-0.005 (0.019) [0.944]	0.007 (0.019) [0.968]
Control Group's Mean	0.037	0.041	0.007	0.015	0.010	0.010	0.024	0.025	0.013	0.017
Anderson-Rubin test (p-value)	0.214	0.332	0.914	0.804	0.066	0.752	0.212	0.549	0.77	0.712
N	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. Stepwise multiple testing p-values, by Romano and Wolf (2015), in brackets. *** p<0.01, ** p<0.05, * p<0.1

Table A15. Impact on Female Empowerment

	Instrumental Variables FE Model Second Stage	
	Decision making at home index (z-score)	
	CM vs. HU	CM vs. NP
Attended CM	0.147 (0.102)	0.004 (0.088)
Control Group's Mean	0.000	0.000
Anderson-Rubin test (p-value)	0.149	0.966
N	5,158	5,128

Notes: CM = randomly encouraged to attend Ciudad Mujer. HU = randomly encouraged to attend a Health Unit. NP = control group with no promotion.

Decision making at home index (woman participates taking decisions about): The children's clothing; The children's education; Health services for the children; Health services for her; Purchase of household assets; Large household repair expenses; If she should work or not.

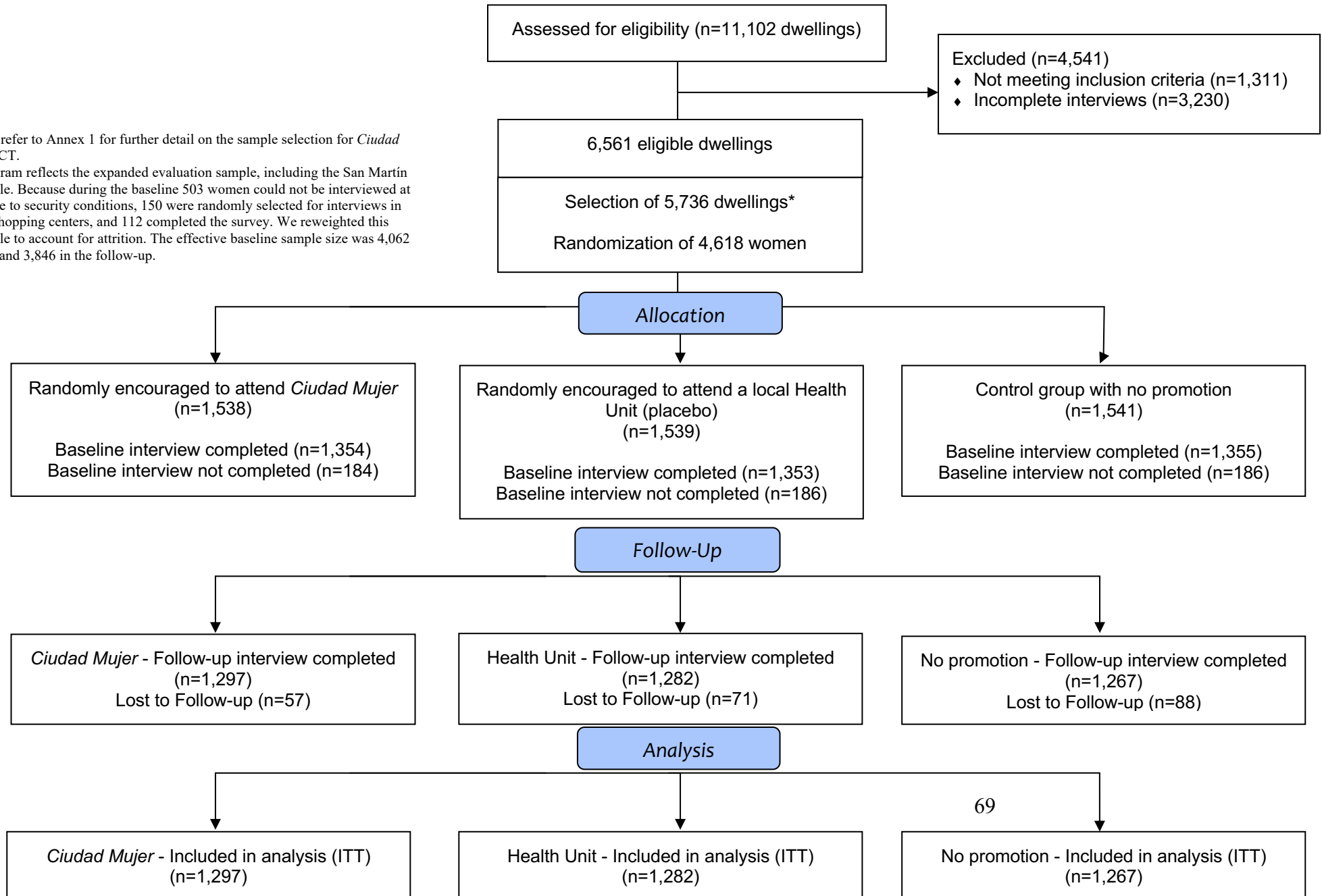
In all 2SLS estimates, the *Promoted to CM* variable instruments for the second-stage dependent variable *Visited CM*.

All observations weighted by the inverse probability of being selected in the sample (IPW).

Standard errors clustered at PSU level in parentheses. *** p<0.01, ** p<0.05, * p<0.1

Ciudad Mujer CONSORT Flow Diagram

* Please refer to Annex 1 for further detail on the sample selection for *Ciudad Mujer* RCT.
The diagram reflects the expanded evaluation sample, including the San Martín subsample. Because during the baseline 503 women could not be interviewed at home due to security conditions, 150 were randomly selected for interviews in nearby shopping centers, and 112 completed the survey. We reweighted this subsample to account for attrition. The effective baseline sample size was 4,062 women, and 3,846 in the follow-up.



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